

Novo Nordisk A/S (NVO)

Long-Form Investment Memo

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Recommendation: SHORT, with a \$37.99 (-19.6%) price target to be reached by year-end 2026, relative to a \$47.24 share price as of August 7th, 2026.

Introduction:

Novo Nordisk is a primarily GLP-1-focused pharmaceutical company that is widely considered one of the two leaders in the global GLP-1 market. Known contemporarily for developing the well-known GLP-1 drugs Ozempic and Wegovy, Novo Nordisk has suffered significant share price decreases within the past two years and has recently released the first-ever weight-loss pill in the US.

Tailwinds:

- **\$30bn to \$100bn Global Market Growth:** The Global Anti-Obesity Market is projected to grow from a current value of \$30bn to at least \$100bn by the early 2030s.
- **Oral GLP-1 Market Dominance:** Novo Nordisk holds ~90% US oral GLP-1 market share, following its first-mover advantage with Oral Wegovy over Eli Lilly's Foundayo.
- **Pipeline Innovation:** Novo Nordisk's current pipeline includes multiple notable drugs with industry-leading qualities in administration and efficacy throughout the weight loss, Type 2 Diabetes, MASH (F1-F4), and MACE risk reduction spaces.

Headwinds:

- **Semaglutide Patent Expirations:** semaglutide, the active ingredient in Ozempic and Wegovy and Novo Nordisk's core revenue driver, has patent expirations occurring in multiple population-heavy countries, including China, India, Canada, and more.
- **New Market Entrants:** While Novo Nordisk and Eli Lilly have long held duopolistic control over the Global GLP-1 market, companies including Pfizer, Roche, Amgen, and more are continuously investing in their own GLP-1 pipeline competitors.
- **Eli Lilly's Retatrutide and Foundayo:** The injectable weight-loss drug Retatrutide and oral drug Foundayo both have leading qualities in efficacy and convenience, respectively, that represent key threats to Novo Nordisk's current and pipeline GLP-1 portfolio.

Business Overview

Novo Nordisk is a Danish pharmaceutical company that specializes in three main disease areas: type 2 diabetes (T2D), obesity, and rare diseases like the bleeding disorder Hemophilia A.¹ As of Q2 2026, the company's leadership, which had multiple new additions throughout 2025 including current CEO Mike Doustdar, has largely divested from several non-core assets like stem cell research to redirect capital towards its core and growing T2D and obesity sectors. However, the company has maintained its commitment to expanding its cardiovascular and chronic liver disease drug profile as well, with drugs including [Ziltivekimab](#), an anti-inflammatory drug for people with atherosclerotic cardiovascular disease (ASCVD), and [Efruxifermin](#), a long-acting FGF21 analog drug that treats a type of fatty liver disease called MASH, both in late-stage Phase 3 clinical trials.²

While previously known as the leading global provider of insulin, Novo Nordisk is known contemporarily as one of two leaders in the US and international GLP-1 markets. The term GLP-1 refers to glucagon-like peptide-1, a naturally produced hormone that stimulates insulin production and slows the digestion of food, making one feel satiated longer.³ While originally produced for the purpose of regulating blood sugar levels for people with type 2 diabetes, GLP-1 drugs that were FDA-approved for treating the disease like Novo Nordisk's

¹ Novo Nordisk, "Q3 Presentation 2025," Investor Presentation: First nine months of 2025, November 5, 2025, <https://investor.novonordisk.com/q3-presentation-2025/>.

² Novo Nordisk, "Q1 2026 Investor Presentation," Investor Presentation: First three months of 2026, May 6, 2026, <https://www.novonordisk.com/content/dam/nncorp/global/en/investors/pdfs/financial-results/2026/Q1-2026-investor-presentation.pdf>.

³ Megan Dorrell, "Rx History: The Rise of GLP-1s," Innovative Rx Strategies (blog), January 25, 2024, <https://innovativerxstrategies.com/rx-history-glp1s/>.

[Ozempic](#) (the injectable T2D brand name for the active ingredient semaglutide) gained widespread interest among especially non-diabetics as stories of the drug's significant weight loss were spread on social media. Subsequently, demand skyrocketed among the American public for a weight-loss-approved GLP-1 like Ozempic.

The FDA approval of semaglutide-based Subcutaneous (SC) [Wegovy](#) for chronic weight management in June of 2021 served as a critical catalyst for both Novo Nordisk's and the broader US GLP-1 market's expansion. While the company had previously released an injectable drug specifically for chronic weight management called [Saxenda](#), which contains the older and comparatively lower-efficacy active ingredient liraglutide, Novo Nordisk had seen comparatively minimal success from the sale of the drug. This was in part due to the drug's once-daily injection frequency and the relatively small weight-loss efficacy that patients derived.⁴ However, by developing a higher-dose version of semaglutide specifically made for chronic weight management (CWM), SC Wegovy reduced the injection frequency to a once-weekly basis and provided consumers with over twice the efficacy of Saxenda.

⁴ Domenica M. Rubino, Frank L. Greenway, and Usman Khalid, "Effect of Weekly Subcutaneous Semaglutide vs Daily Liraglutide on Body Weight in Adults with Overweight or Obesity without Diabetes: The STEP 8 Randomized Clinical Trial," JAMA, January 11, 2022, <https://jamanetwork.com/journals/jama/fullarticle/2787907>.

Figure 1.1 – Prominent GLP-1 Drugs:

	DOSAGE	DOSAGE FORM	APPROVED FOR	WHO CAN TAKE IT?	OTHER BENEFITS
Ozempic (SEMAGLUTIDE)	1 WEEKLY		TYPE 2 DIABETES	 ADULTS	HEART, KIDNEYS, WEIGHT LOSS
Rybelsus (SEMAGLUTIDE)	1 DAILY		TYPE 2 DIABETES	 ADULTS	WEIGHT LOSS
Wegovy (SEMAGLUTIDE)	1 WEEKLY		WEIGHT LOSS	12+  KIDS + ADULTS	N/A
Trulicity (DULAGLUTIDE)	1 WEEKLY		TYPE 2 DIABETES	10+  KIDS + ADULTS	HEART, KIDNEYS, WEIGHT LOSS
Victoza (LIRAGLUTIDE)	1 DAILY		TYPE 2 DIABETES	10+  KIDS + ADULTS	HEART, KIDNEYS, WEIGHT LOSS
Saxenda (LIRAGLUTIDE)	1 DAILY		WEIGHT LOSS	12+  KIDS + ADULTS	N/A
Byetta (EXENATIDE)	2 DAILY		TYPE 2 DIABETES	 ADULTS	WEIGHT LOSS
Bydureon BCise (EXENATIDE)	1 WEEKLY		TYPE 2 DIABETES	10+  KIDS + ADULTS	WEIGHT LOSS
Mounjaro (TIRZEPATIDE)	1 WEEKLY		TYPE 2 DIABETES	 ADULTS	WEIGHT LOSS

(A comparison of prominent GLP-1 drugs and their characteristics. Ozempic, Wegovy, and Rybelsus are all brand names for the same active ingredient, semaglutide, produced by Novo Nordisk.)

As a result of the drug's efficacy and administration superiority over its older counterpart, Wegovy's demand skyrocketed to levels far exceeding what Novo Nordisk had expected. The drug manufacturer had prepared for product demand similar to what ensued following the launch of Saxenda and was left unprepared when Wegovy surpassed Saxenda's five-year prescription rate in only five weeks.⁵ As Novo Nordisk faced short-term supply constraints, representatives from the company instructed doctors nationwide to stop new Wegovy prescriptions for patients.⁶ While the insatiable post-launch demand from patients unable to get an initial Wegovy prescription was partially quenched by doctors who prescribed Ozempic off-label to non-

⁵ Peter Loftus and Noemie Bisserbe, "How Ozempic's Maker Lost Its Grip on the Obesity Market It Created," The Wall Street Journal, May 28, 2025, <https://www.wsj.com/health/pharma/novo-nordisk-ozempic-anti-obesity-drug-market-5d5194a6>.

⁶ Alyssa Billingsley, "Is Semaglutide in Shortage? The Latest on Ozempic and Wegovy Availability," GoodRx, August 18, 2025, <https://www.goodrx.com/classes/glp-1-agonists/semaglutide-shortage>.

diabetics, a large portion of consumers were left with their GLP-1 needs unmet.⁷ This strong demand for Wegovy and Ozempic initially allowed Novo Nordisk to consolidate a majority of the US and Global GLP-1 obesity and T2D markets, but also caused the widespread dearth of semaglutide-based products produced by the company. As a result, the FDA added Wegovy and Ozempic to its national drug shortage list in March and August of 2022, respectively.⁸ This classification subsequently allowed compounding companies, currently one of Novo Nordisk's largest rivals in the lucrative US GLP-1 market, to create and sell one-to-one copies of the brand-name drugs legally.⁹

Although the shortage for both drugs was officially ended in February 2025, compounders and telehealth companies that produce their own semaglutide-based GLP-1s have found ways to skirt FDA regulation and continue to produce cheaper versions of weight-loss and T2D drugs.¹⁰ The Federal Food, Drug and Cosmetic Act allows compounders to continue producing compounded versions of patented GLP-1s if the drugs create a significant difference for the patient when compared to the brand-name version they are based on.¹¹ Compounders

⁷ John P.H. Wilding, Rachel L. Batterham, and Salvatore Calanna, "Once-Weekly Semaglutide in Adults with Overweight or Obesity," *New England Journal of Medicine*, February 21, 2021, <https://www.nejm.org/doi/pdf/10.1056/nejmoa2032183>.

⁸ U.S. Food and Drug Administration, "Drug Shortages," content current as of July 15, 2026, <https://www.fda.gov/drugs/drug-safety-and-availability/drug-shortages>.

⁹ Center for Drug Evaluation and Research, "Compounding When Drugs Are on FDA's Drug Shortages List," U.S. Food and Drug Administration, August 8, 2025, <https://www.fda.gov/drugs/human-drug-compounding/compounding-when-drugs-are-fdas-drug-shortages-list>; Center for Drug Evaluation and Research, "FDA Clarifies Policies for Compounders as National GLP-1 Supply Begins to Stabilize," U.S. Food and Drug Administration, April 28, 2025, <https://www.fda.gov/drugs/drug-safety-and-availability/fda-clarifies-policies-compounders-national-glp-1-supply-begins-stabilize>.

¹⁰ FDA Center for Drug Evaluation and Research, "FDA Clarifies Policies for Compounders as National GLP-1 Supply Begins to Stabilize," U.S. Food and Drug Administration, April 28, 2025, <https://www.fda.gov/drugs/drug-safety-and-availability/fda-clarifies-policies-compounders-national-glp-1-supply-begins-stabilize>.

¹¹ U.S. Department of Health and Human Services Food and Drug Administration, "Compounded Drug Products That Are Essentially Copies of a Commercially Available Drug Product Under Section 503A of the Federal Food, Drug, and Cosmetic Act," FDA.gov, January 2018, <https://www.fda.gov/files/drugs/published/Compounded-Drug-Products-That-Are-Essentially-Copies-of-a-Commercially-Available-Drug-Product-Under-Section-503A-of-the-Federal-Food--Drug--and-Cosmetic-Act-Guidance-for-Industry.pdf>.

have used this regulatory loophole to create patient-tailored GLP-1s that include randomized amounts of additives, usually various types of vitamins, that they claim constitute a significant difference from the brand-name versions of semaglutide.¹² While the FDA issued over 100 warning letters to telehealth companies and compounders regarding false and misleading product differentiation claims throughout 2025, consumer demand for the cheaper drugs has led many compounders to continue their operations.¹³

Nevertheless, there has been an increase in compounder crackdowns by the FDA and litigation efforts by Novo Nordisk against prominent compounders and telehealth companies since the start of 2026. In February, the FDA announced it would take steps to restrict non-approved active pharmaceutical ingredients (APIs) used by telehealth companies and specifically named Hims & Hers as a key telehealth compounder subject to these restrictions.¹⁴ Novo Nordisk sued Hims & Hers soon after the FDA statement for infringement on its semaglutide patent and deceptive marketing, leading to the two companies settling and partnering. Hims & Hers subsequently exited the compounded GLP-1 space and became one of Novo Nordisk's largest direct-to-consumer partners. However, compounding companies were not the only threat that developed out of the semaglutide shortages. Another, even more substantial threat, rose from Novo's capacity constraints that eventually evolved into what is the largest direct competitor to Novo Nordisk contemporarily: Eli Lilly.

¹² Alex Janin, "Ozempic Knockoffs Survive Crackdown Thanks to Loophole," The Wall Street Journal, May 18, 2025, <https://www.wsj.com/health/pharma/ozempic-knockoffs-survive-crackdown-thanks-to-loophole-7c5b02d7>.

¹³ U.S. Food and Drug Administration, "Warning Letters," last modified September 25, 2025, <https://www.fda.gov/inspections-compliance-enforcement-and-criminal-investigations/compliance-actions-and-activities/warning-letters>.

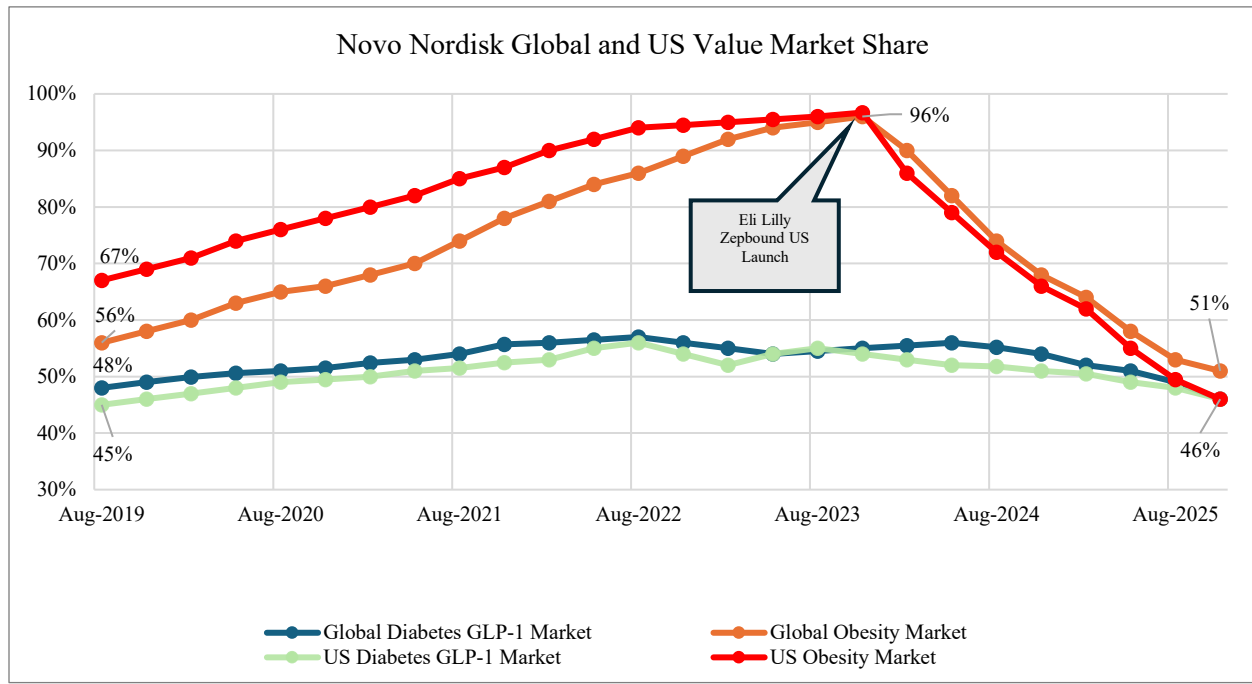
¹⁴ U.S. Food and Drug Administration, "FDA Intends to Take Action Against Non-FDA-Approved GLP-1 Drugs," press release, February 6, 2026, <https://www.fda.gov/news-events/press-announcements/fda-intends-take-action-against-non-fda-approved-glp-1-drugs>.

The robust weight-loss GLP-1 demand that remained in the market, even with both official and compounded semaglutide being produced, allowed the American pharmaceutical company Eli Lilly to cannibalize Novo Nordisk's US and global GLP-1 market share with its two principal GLP-1 drugs: the weight-loss drug [Zepbound](#) and T2D drug [Mounjaro](#). The reason that Eli Lilly's drugs, which use the active ingredient tirzepatide, were able to end Novo Nordisk's near-monopoly on the US GLP-1 industry was due to one simple factor: efficacy. Both of Eli Lilly's drugs are dual agonists, meaning they activate both the GLP-1 and GIP (glucose-dependent insulintropic polypeptide) receptors throughout the body. This critical difference between tirzepatide and semaglutide, a GLP-1 mono-agonist, is the reason the former promotes significantly higher weight-loss and HbA1c (a measure of blood sugar) reduction efficacy in patients than semaglutide does.¹⁵ This factor alone led the two drugs to effectively propel Lilly into becoming the first healthcare company to reach a \$1 trillion market cap in November of 2025.¹⁶ As a result of Lilly's drugs, Novo has faced extreme competition and mounting pressure from the rise of Lilly's two GLP-1 portfolio competitors. With Eli Lilly and compounding companies overperforming on drug efficacy and undercutting on drug prices, respectively, Novo Nordisk has faced an attack on its once domineering GLP-1 franchise that has contributed heavily to the company's diminishing share price and financial performance within the past two years.

¹⁵ Patricia J. Rodriguez et al., "Semaglutide vs Tirzepatide for Weight Loss in Adults With Overweight or Obesity," *JAMA Internal Medicine* 184, no. 9 (2024): 1056–64, <https://doi.org/10.1001/jamainternmed.2024.2525>.

¹⁶ Angel Adegbesan and Avalon Pernell, "Lilly Joins \$1 Trillion Club in Weight-Loss Drug Fueled Climb - Bloomberg," Bloomberg, November 21, 2025, <https://www.bloomberg.com/news/articles/2025-11-21/lilly-joins-1-trillion-club-in-weight-loss-drug-fueled-climb>.

Figure 1.2 – Novo Nordisk TRx Market Share:



(A graph depicting the value market share, meaning the share of total market revenue, of Novo Nordisk in four key markets. It should be noted that between August 2019 and August 2022, Novo Nordisk’s reported US value market share for both diabetes and obesity represents the entire North American sector; while US diabetes and obesity value market share for August 2022 and after represent US value market share solely.)

Contemporarily, Novo Nordisk has borne the brunt of its crucial misstep in preparing for the launch of Wegovy, with the company’s New York Stock Exchange depositary receipt NVO falling about 70% from its peak share price in June 2024 to August 2026.¹⁷ Its principal competitor, Eli Lilly, has hit consecutive all-time highs over the same time period, with its stock rising about 50% in the past year alone.¹⁸ Figure 1.2 highlights cannibalization that Novo Nordisk experienced from the release of Eli Lilly’s Zepbound and Mounjaro, especially within

¹⁷ Yahoo! Finance, “Novo Nordisk A/S (NVO) Stock Price, News, Quote & History,” Yahoo! Finance, <https://finance.yahoo.com/quote/NVO/>.

¹⁸ Yahoo! Finance, “Eli Lilly and Company (LLY) Stock Price, News, Quote & History,” Yahoo! Finance, <https://finance.yahoo.com/quote/LLY/>.

the US and global obesity markets where Zepbound has dominated Wegovy in total prescriptions (TRx).¹⁹

Contemporarily, market sentiment regarding Novo Nordisk is at near all-time lows. The share price dropped 14% to \$44.28 from July 30th to August 4th due to two back-to-back events: the company announcing that its ZEUS trial, which measured Ziltivekimab 15 mg's effectiveness in reducing the risk of Major Adverse Cardiovascular Events (MACE), failed to achieve its primary risk reduction goals as well as underwhelming results in Novo's Q2 2026 report for the company's new oral weight-loss pill [Oral Wegovy](#).²⁰ Although the stock recovered some of its losses, hovering at \$47.24 as of Friday August 7th, it remains near its 5-year low of just above \$35 which it reached in late March of 2026 following another key drug efficacy clinical trial failure with [CagriSema](#). In that case, management had continuously stated their belief in the possibility of a 25% weight-loss efficacy from the drug and ultimately let investors down when the drug failed to reach the primary endpoint of showing non-inferiority to Eli Lilly's tirzepatide-based drugs.²¹ With the failure of the ZEUS trial effectively eliminating Novo Nordisk's ability to capitalize on the \$3-\$5 billion Atherosclerotic Cardiovascular Disease (ASCVD) and Chronic Kidney Disease (CKD) market in the near term, there remain fewer tailwinds supporting the company in its attempt to regain market dominance.²²

¹⁹ Emmanuel Papadakis et al., "Weekly retail Rx tracker," Deutsche Bank Research, July 13, 2026. [Restricted-access sell-side research.]

²⁰ Novo Nordisk, "Novo Nordisk Provides Update on the ZEUS Phase 3 Trial in People with ASCVD, CKD and Inflammation," press release, July 31, 2026, <https://www.novonordisk.com/content/nncorp/global/en/news-and-media/news-and-ir-materials/news-details.html?id=916587>.

²¹ Novo Nordisk, "Novo Nordisk A/S: CagriSema demonstrated 23% weight loss in an open-label head-to-head REDEFINE 4 trial in people with obesity, the primary endpoint was not achieved," press release, February 23, 2026, <https://www.novonordisk.com/content/nncorp/global/en/news-and-media/news-and-ir-materials/news-details.html?id=916501>.

²² Thibault Bouterin et al., "US readout: a thunderbolt from ziltivekimab?," Morgan Stanley, July 15, 2026. [Restricted-access sell-side research.]

The primary thesis drivers built into the company's current valuation include the outcomes of the Phase 3 SYNCHRONY Real-World safety trial for Efruxifermin in late 2026, the launch and performance of upcoming pipeline GLP-1s including CagriSema and [Zenagamtide](#), and the future trend in TRx for Novo's flagship GLP-1 drugs Ozempic and Wegovy as semaglutide experiences patent loss of exclusivity (LOE) in several countries starting this year. However, as the company grapples with losing patent protection on the active ingredient that made up 70% of its Quarter 1 2026 top-line sales, investors have increasingly relied on one principal aspect above all else as a sign of Novo's future financial prospects: the performance of its new weight-loss drug Oral Wegovy.

Receiving FDA approval in December of 2025 and launching within the US in January of this year, the pill represented a major step in pharmacology as the first ever oral formulation approved for chronic weight management. The sentiment regarding Novo Nordisk skyrocketed around the prospects of Oral Wegovy, with the company's stock rising 33% in the month following its US approval before erasing those gains following management's bleak 2026 financial outlook.²³ The drug's first-mover advantage was soon ended, however, with the FDA approval and US launch of Eli Lilly's weight-loss pill [Foundayo](#) in April of 2026, which currently represents the biggest threat to Novo's stock recovery. Therefore, Novo's success and future share price performance significantly depends on whether it can maintain its three-month lead on Lilly's oral competitor and continue to scale Oral Wegovy or whether the drug will experience the same fate as Ozempic and Wegovy. Furthermore, while the company's core revenue drivers will change dramatically in the coming years as the incumbent semaglutide

²³ Novo Nordisk, "Q4 2025 Novo Nordisk A/S Earnings Conference Call," webcast, February 4, 2026, <https://edge.media-server.com/mmc/p/4kfpu8op/>.

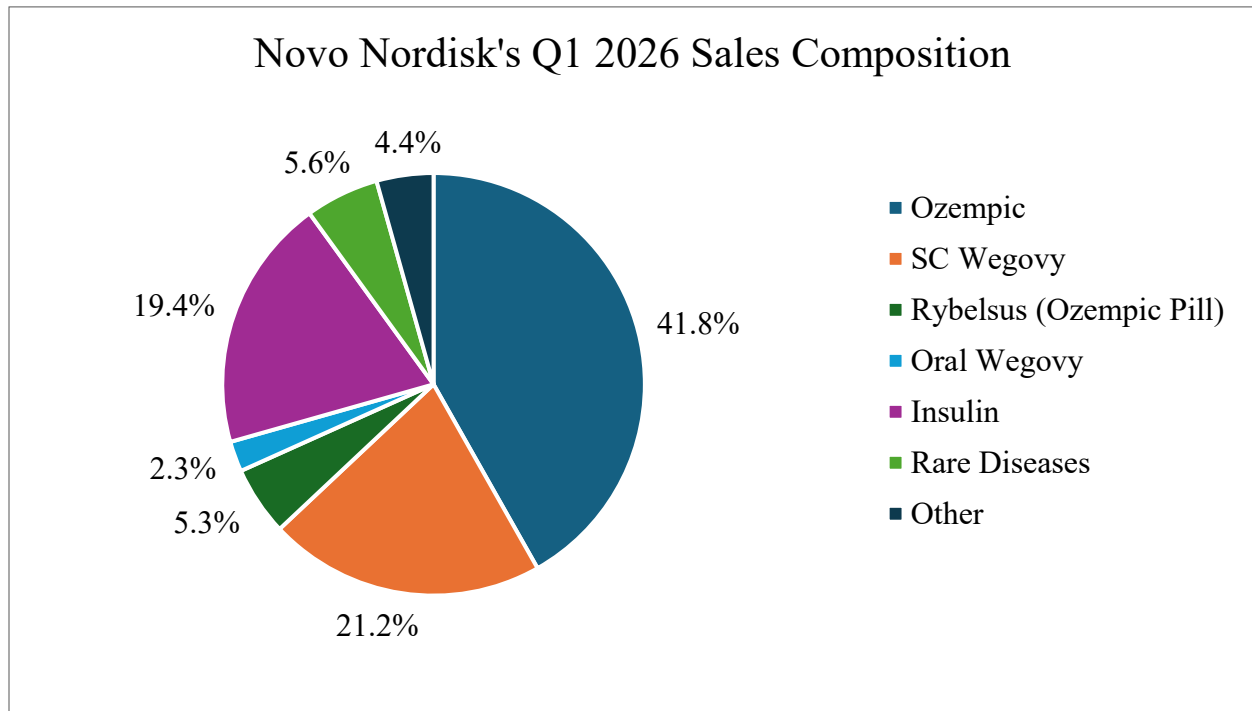
injections phase out and new pipeline drugs are approved, Novo's current revenue drivers remain primarily within its GLP-1 portfolio.

Core Revenue Drivers

As of Quarter 1, 2026, Novo Nordisk's core revenue drivers largely remain its T2D and weight-loss drugs, primarily the injectable drugs Ozempic and Wegovy and their oral alternatives [Rybelsus](#) and Oral Wegovy. Figure 2.1 shows the exact top-line contributions of each drug, with Ozempic and SC Wegovy alone representing over 60% of the company's heavily concentrated quarterly revenue. Furthermore, while the patents for semaglutide expired in several countries in 2026, a key patent cliff date for Novo Nordisk is the US LOE for the active ingredient in December of 2031. While Novo Nordisk is a Danish company headquartered in Denmark that sells its drugs globally, the US consistently remains its most lucrative market, representing 56% of all sales for the full-year 2025. Furthermore, while semaglutide-based drugs represented 73.9% of the company's total full-year 2025 sales, sales of those same drugs solely within the US accounted for 48% of Novo's full-year 2025 sales. This extreme reliance on the US therefore makes the 2031 patent expiration date one of Novo Nordisk's steepest projected revenue drop-off periods, given that sales retention for originator brand-name drugs following generic drug entry has historically declined to 73.1% after one year and to 31.7% by year five.²⁴

²⁴ Soon Kyu Jung and Sang-Won Lee, "Sales Erosion of Originator Drugs Following Generic Entry: Quantitative Analysis and Predictive Modeling," *Therapeutic Innovation & Regulatory Science* 60, no. 1 (January 2026): 248–259, <https://pubmed.ncbi.nlm.nih.gov/41166003/>.

Figure 2.1 – Novo Nordisk Q1 Contributions:



(A chart showing the percentage of Novo Nordisk's Quarter 1 2026 sales revenue that key drugs and drug groups comprised)

As stated above, however, there are several pipeline and currently available drugs that one would be remiss not to consider as future core revenue drivers. To start, Oral Wegovy is widely considered to be Novo Nordisk's next core revenue driver for several reasons, primarily cost and administration. The lower-cost and oral aspects of the drug and its T2D counterpart Rybelsus, which is actively being retired and replaced by the essentially identical Ozempic Pill, remain the reasons for investors' reliance on their future performance, especially Oral Wegovy. Nevertheless, there exist several opportunities in Novo's injectable portfolio for future core revenue drivers, including CagriSema, the injectable combination weight-loss drug that mixes the insulin secretion properties of the GLP-1 receptor-activator semaglutide with the satiety-enhancing properties of the amylin and calcitonin receptor-activator cagrilintide. Blending these

two active ingredients has allowed CagriSema to produce an average weight-loss efficacy close to 23%, higher than any currently marketed weight-loss or T2D GLP-1 drug produced by Novo Nordisk, although crucially still below Eli Lilly's tirzepatide-based dual-agonist Zepbound.²⁵

Another pipeline drug that shows even higher potential than CagriSema is Zenagamtide, a unimolecular dual agonist drug that activates the GLP-1 and amylin receptors. This first-in-class drug would also notably come in two forms of administration: a once-weekly injection and a once-daily oral tablet, with clinical trials for the injection showing 23.2% placebo-adjusted weight loss efficacy after 36 weeks among overweight or obese individuals.²⁶ Additionally, Zenagamtide shows potential to serve as a T2D medication, reducing HbA1c up to 1.6% placebo-adjusted in clinical trials. This duality means that the drug could serve as Novo Nordisk's next-generation version of Wegovy and Ozempic with higher weight-loss efficacy and significant blood sugar regulation qualities, provided that future clinical trial results reaffirm or improve past efficacy.²⁷ It is important to note the current timeline for the drug's release, however, specifically given multiple Phase 3 trials analyzing subcutaneous and oral Zenagamtide efficacy have 2028 end times, meaning Zenagamtide likely won't be FDA-approved and commercially available in the US until at least 2029.

²⁵ American Diabetes Association, "CagriSema Demonstrates Significant Weight Loss in Adults with Obesity," CagriSema Demonstrates Significant Weight Loss in Adults with Obesity | American Diabetes Association, June 22, 2025, <https://diabetes.org/newsroom/press-releases/cagrisema-demonstrates-significant-weight-loss-adults-obesity>.

²⁶ Novo Nordisk Inc., "Novo Nordisk Advances Early-Stage Obesity Medication, Amycretin, to Phase 3 Clinical Development Based on Early-Phase Clinical Trial Results in People with Obesity or Excess Weight, Published in the Lancet," PR Newswire: press release distribution, targeting, monitoring and marketing, June 20, 2025, <https://www.prnewswire.com/news-releases/novo-nordisk-advances-early-stage-obesity-medication-amycretin-to-phase-3-clinical-development-based-on-early-phase-clinical-trial-results-in-people-with-obesity-or-excess-weight-published-in-the-lancet-302487500.html>.

²⁷ Luke Halpern, "Amycretin Shows Significant Weight Loss, HbA1c Reductions in Type 2 Diabetes," Pharmacy Times, January 15, 2026, <https://www.pharmacytimes.com/view/amycretin-shows-significant-weight-loss-hba1c-reductions-in-type-2-diabetes>.

In summary, Novo Nordisk's current core revenue drivers are largely their principal two GLP-1 drugs, SC Wegovy and Ozempic, especially within the US. Oral Wegovy and its competitor Foundayo represent novel steps forward in the GLP-1 weight-loss space, while CagriSema and Zenagamtide remain prominent pipeline drugs that have the potential to become core revenue drivers for Novo Nordisk in the coming years. As a result, the company is heavily reliant on the future success, both in clinical trial results and TRx performance/retention, of these core drugs as it faces continuously declining market share to Eli Lilly's weight-loss superiority and compounding companies' price advantage. It is within this reliance, specifically on Oral Wegovy in the short term, that I argue there exists a financial opportunity in the drug's future underperformance.

Thesis

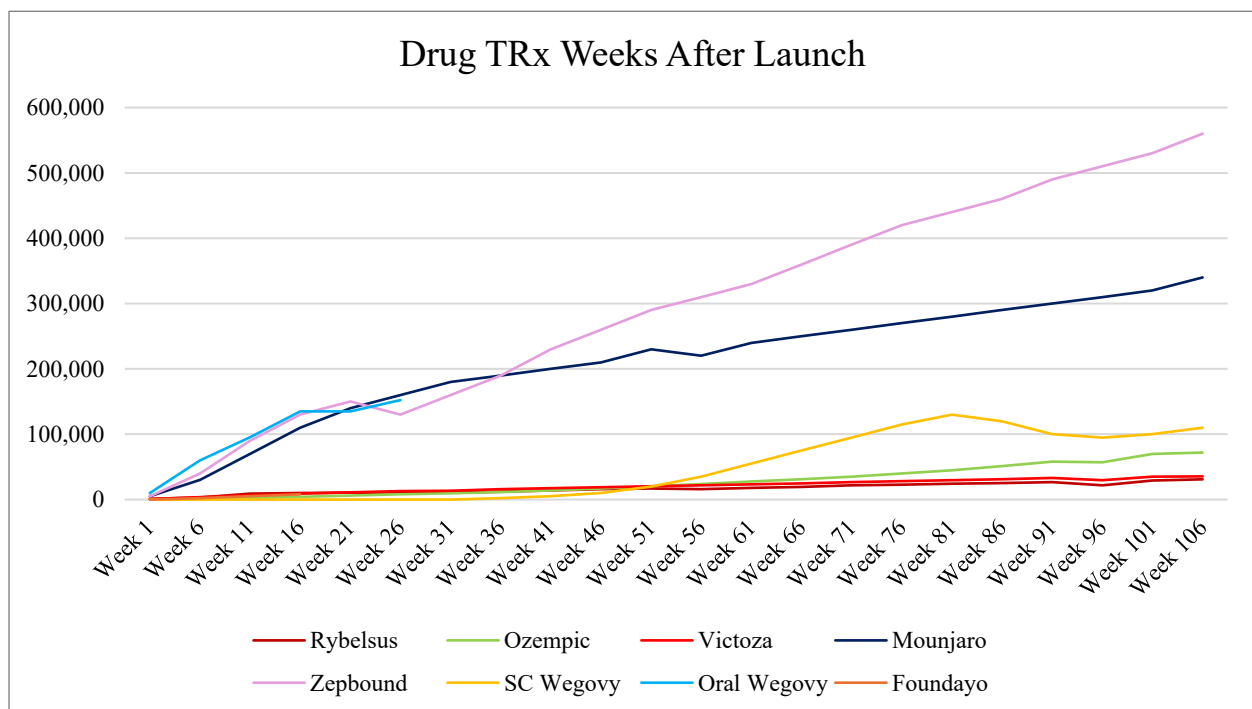
Foundayo Supersedes Oral Wegovy as the Dominant Oral GLP-1

Introduction

With the US FDA approval and commercial launch of Ozempic in late 2017 and early 2018, respectively, the GLP-1 industry entered a structurally different era for treating chronic weight loss in overweight individuals and regulating blood sugar levels in type 2 diabetics. Previous-generation GLP-1s, which I classify as those released before Ozempic, had several disadvantages that alienated consumers from using them as fervently as what drove the semaglutide shortage following Wegovy's release in the summer of 2021. Previous-generation

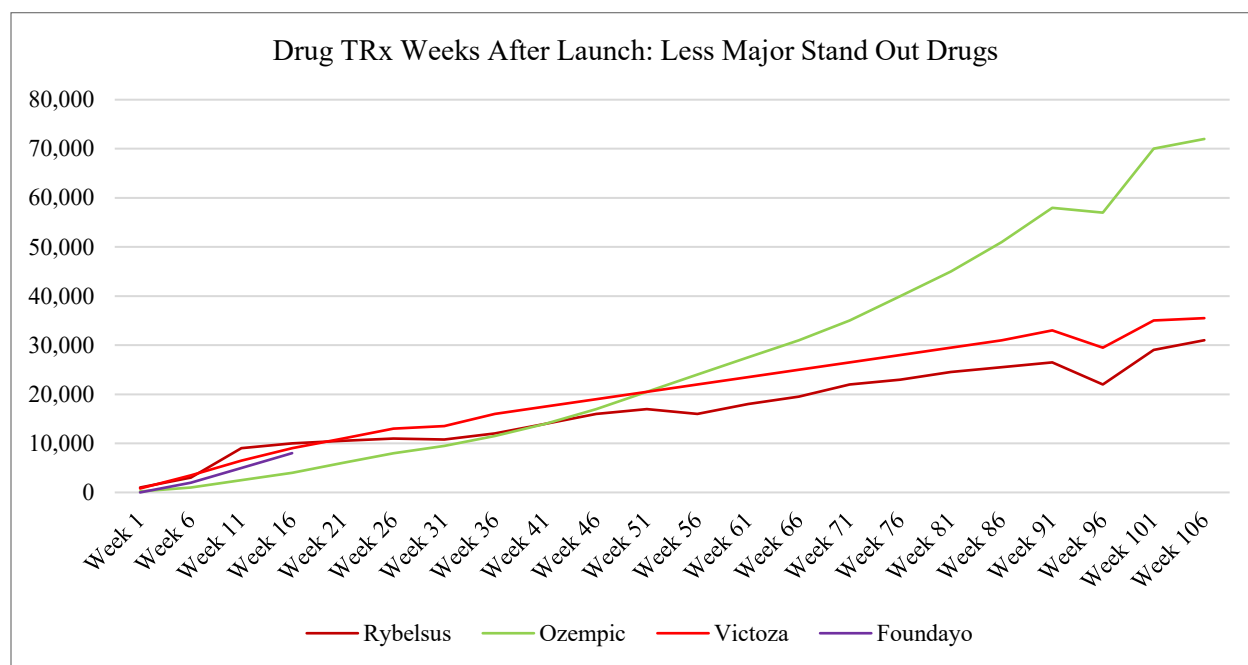
T2D GLP-1s included Eli Lilly’s co-promoted [Byetta](#), which launched in 2005 as the first-ever GLP-1 and required two time-restricted subcutaneous injections daily for relatively moderate HbA1c reduction, and Novo Nordisk’s [Victoza](#), which launched in 2010 and required a slightly less burdensome once-daily subcutaneous injection with marginally better HbA1c reduction. Nevertheless, while these administration frequencies seem tedious given the current state of T2D injectables, the medical advancements these two drugs brought should not be understated. Compared to Novo Nordisk’s previous T2D injectable insulin offerings, including Levemir and Novolog, which required at least daily injections and, at most, injections before every major meal of the day, the reduction in administration frequency and the increase in A1c-regulating efficacy were pivotal new offerings for consumers. Rybelsus, the only currently available oral T2D treatment, took this a step further by providing even higher efficacy than Byetta or Victoza and eliminating the injection aspect of administration.

Figure 3.1 - Prominent GLP-1s TRx After Launch:



(A graph showing the TRx after launch of past- and current-generation T2D and Weight-loss drugs. Key drugs include Novo Nordisk's Oral Wegovy in light blue and Eli Lilly's Foundayo pill in orange. Oral Wegovy TRx cites IQVIA data rather than management's internal numbers to provide a more apples-to-apples growth rate comparison between brands.)

Figure 3.2 – Prominent GLP-1s Less Stand Out TRx After Launch:



(A condensed graph showing the TRx after launch of past- and current-generation T2D and Weight-loss drugs, less major blockbusters including Mounjaro, Zepbound, Oral Wegovy, and SC Wegovy.)

Compared to the two-decade history of GLP-1 T2D treatments and their transition from daily injections to daily pills, GLP-1 weight-loss treatments are a relatively new development. While drugs like Byetta provided marginal weight loss for the T2D patients who used it, the first weight-loss-approved GLP-1 was commercially available almost 10 years after Byetta. Released in early 2015 for chronic weight management, Saxenda marked the beginning of the GLP-1 weight-loss industry, providing a 4.4% placebo-adjusted weight loss efficacy in exchange for a once-daily injection administration requirement. Saxenda would remain the only weight-loss-approved GLP-1 until Novo Nordisk released Wegovy in 2021, which saw comparatively eclipsing consumer demand. Figures 3.1 and 3.2 show the TRx for several major drugs following their US commercial launch. While SC Wegovy initially showed meager TRx because of the

major supply constraints that Novo Nordisk experienced following the drug's release, the growth beginning week 52 represented the strong backlog of consumer demand that had remained for the weight-loss GLP-1 even after a year.

Efficacy Comparison

Given that I define current-generation drugs as Ozempic, Wegovy, Zepbound and Mounjaro and using TRx as the closest proxy for true consumer demand available, what these two figures show is an extreme increase in consumer demand for current-generation drugs relative to their past-generation counterparts. Consumer demand for Wegovy relative to Saxenda reached over 4 times by week 81 while demand for Ozempic relative to its injectable counterpart Victoza reached a more moderate 1.5 times by the same time after launch. However, Eli Lilly's comparable drugs dominated the current generation improvements that Novo Nordisk launched, with its weight-loss drug Zepbound reaching over 3 times the TRx level of SC Wegovy by week 81 and its T2D drug Mounjaro reaching over 6 times the TRx compared to Ozempic by the same point. Through these comparisons in consumer demand relative to the same period following each drug's launch, one can identify the key driver that induced soaring consumer demand for Novo Nordisk's current-generation drugs over its past-generation counterparts and for Eli Lilly's current-generation drugs over Novo's. It's the factor most would expect, and the factor that Novo's own management has continuously cited as being a key driver for Oral Wegovy's success over Eli Lilly's Foundayo. Most of all, it's the factor that I argue consumers will increasingly prioritize less in the coming months: Efficacy.

When comparing the A1c reduction efficacy of Mounjaro versus Ozempic, the former beats out the latter at each comparable dosage and trial length: Mounjaro's 5 mg, 10 mg, and 15 mg doses compared to Ozempic's 0.5 mg, 1 mg, and 2 mg doses at 40-56 weeks provide 0.69%, 0.38%, and 0.1% more A1c reduction. The same comparison exists in the two companies' weight-loss drugs: over 72 weeks, Zepbound's 5 mg, 10 mg, and 15 mg doses provide 11.9%, 16.4%, and 17.8% placebo-adjusted weight loss efficacy relative to Wegovy's standard 2.4 mg and 7.2 mg doses, which provide 11.7% and 14.8% placebo-adjusted weight loss efficacy. While there may be many other structural tailwinds that would push Eli Lilly's drugs ahead of Novo Nordisk's, the latter company's drugs have had substantial tailwinds as well. From allocating over twice the amount of marketing spend to Ozempic and Wegovy than Eli Lilly did for Zepbound and Mounjaro in the first 9 months of 2025, having arguably the most prominent brand name backing in history, and maintaining a roughly four-and-a-half-year lead and two-and-a-half-year lead in T2D and weight loss, respectively, Novo Nordisk has had several advantages over its American-based competitor and yet still failed to hold on to its market share in both the US and IO GLP-1 markets.²⁸

Furthermore, given that each drug's placebo-adjusted tolerability is in line with its counterpart, the surging consumer demand and market-share cannibalization of Eli Lilly's tirzepatide-based drugs can understandably be attributed to their superior A1c reduction and weight-loss efficacy. Nevertheless, this does not mean that efficacy will continue to be the determining factor for consumers deciding which drug is right for them. Rather, with the recent clinical data shown for weight-loss efficacy in certain drugs, I argue that the consumer will

²⁸ Harshita Mary Varghese and Patrick Wingrove, "Exclusive-Novos Wegovy and Ozempic US Advertising Spend Doubles Rival Eli Lilly, Data Shows," *Reuters*, January 28, 2026, <https://www.reuters.com/business/media-telecom/novos-wegovy-ozempic-us-advertising-spend-doubles-rival-eli-lilly-data-shows-2026-01-28/>.

undergo a structural shift with the onset of the oral GLP-1 market. Rather than prioritizing the total amount of weight loss or A1c reduction a drug can provide in the short term, consumers will increasingly make their healthcare decisions based upon which drug can provide results that are maintainable in the long run and are easiest to integrate into their daily lifestyles.

Thesis Rationale

One of the most prominent pipeline drugs currently in phase 3 trials is Eli Lilly's [Retatrutide](#), a weight-loss and T2D drug that provides the highest efficacy achieved in history with a GLP-1 drug. The drug's weight-loss of 26.1% placebo-adjusted at 80 weeks is so profound it rivals that obtained by a sleeve gastrectomy, a procedure that includes removing 75% to 80% of a patient's stomach to limit the amount of food that can be held and lower hunger hormones.²⁹ The drug achieves this incredible feat by using a triple agonist structure, activating the GLP-1, Glucagon, and GIP receptors to stimulate superior weight-loss efficacy than its dual-agonist or mono-agonist counterparts. Given that the drug provides such high levels of weight loss over such a short period of time, I argue that the drug itself marks the end of efficacy as the dominant factor, and a switch by consumers to favoring a drug's tolerability and lifestyle integrability, especially within the oral GLP-1 market. Furthermore, I contend this trend will materialize for the first time in Founday's TRx overtaking Oral Wegovy's total prescriptions by year-end 2026. My rationale for this switch comes from a two-pronged approach of considering the average weight-loss GLP-1 user and their goals for using a drug like Wegovy or Zepbound over more

²⁹ Cleveland Clinic, "Bariatric Surgery: What It Is, Types & Requirements," *Cleveland Clinic Health Library*, December 17, 2025, <https://my.clevelandclinic.org/health/treatments/bariatric-surgery>.

traditional methods in addition to analyzing current consumer demand trends for oral weight-loss drugs relative to their injectable counterparts.

Regarding the former rationale, I believe the typical reason for using a weight-loss GLP-1 and an oral weight-loss GLP-1 specifically favors Foundayo taking over as the dominant weight-loss pill in the near term. Fundamentally, while the reasons for using a weight-loss GLP-1 vary, the goal for every single patient using Saxenda, Wegovy, Zepbound, or their oral counterparts is the same: to lose weight. According to a study conducted by Prime Therapeutics and reported by Blue Cross Blue Shield Kansas, the 1-year persistence for GLP-1 products among those with obesity without diabetes was 63%, while the 3-year persistence rates dropped to just 8%.³⁰ While the exact reasoning for this plummet in persistence differs from patient to patient, some of the most prominent ones remain physical weakness, frailty, and sluggishness caused by GLP-1's ability to cause up to 10% lean muscle mass loss, comparable to a decade or more of aging.³¹ As a result of this fact, I argue many patients end their GLP-1 usage specifically for weight loss due to the side effects listed above, which are typically more prominent the more weight-loss efficacy a drug promotes. As a result, while high-efficacy drugs like Retatrutide may see initial soaring demand, the resulting increased chance and severity of frailty and other side effects will likely lead to a lower long-term persistence rate for the average GLP-1 user. Due to this, I foresee the highest and, more importantly, most durable demand residing not in the highest efficacy drugs as

³⁰ Blue Cross and Blue Shield of Kansas, "Prime Therapeutics' Leading Research Shows Only 1 in 12 Remain on a GLP-1 Drug for Obesity at Three Years," last updated January 9, 2026, <https://www.bcbsks.com/employers/resources/prime-therapeutics-leading-research-shows-only-1-12-remain-glp-1-drug-obesity-three-years-obesity>.

³¹ Natasha Dangoor, "Is the Weight-Loss Drug Revolution Causing a Frailty Epidemic?," *Wall Street Journal*, updated May 26, 2026, <https://www.wsj.com/health/pharma/glp-1-weight-muscle-loss-frailty-ca277a24>.

has been the case historically, but instead in the moderate-efficacy drugs that provide the least negative side effects and restrictions on patients' daily lives.

This leads me into my second point, which involves analyzing the historical TRx data I have access to and using it as a proxy for consumer demand. While many drugs and their post-launch TRx are shown in Figure 3.1, three drugs stand out in particular: the injectable drugs Zepbound and Mounjaro and the oral drug Oral Wegovy. Notably, based on the most up-to-date TRx data available, one can see that Oral Wegovy demonstrates initial consumer demand largely superior to that of Zepbound or Mounjaro. There are several reasons for why this could be the case, most notably the fact that it's the first oral weight-loss drug, but it nevertheless is a sign of consumer demand for the oral drug being stronger than its injectable counterpart Zepbound or SC Wegovy. Rather than attributing the superior performance to its first-mover advantage or the drug novelty, however, I argue Oral Wegovy's performance thus far is a preliminary sign of the shift in consumer preferences starting to occur. After all, the fundamental reason for using an oral drug rather than an injectable drug like Zepbound or SC Wegovy, which both have much higher weight-loss efficacies, is for one of two principal reasons: a deprioritization of weight-loss efficacy as the deciding factor or an increasing prioritization of the ease of administration and, resultingly, long-term viability of the drug.

I argue that rather than a simple black-or-white factor between these two reasons, the principal rationale for using an oral drug and what is largely driving Oral Wegovy's strong consumer demand currently is a combination of both reasons. Therefore, given this assumption, the key determinant for whether Oral Wegovy will hold on to the superb consumer demand it has demonstrated or whether Foundayo, which Eli Lilly has stated will begin to receive massive marketing support in Quarter 3 of 2026, will cannibalize its market share lies in which drug

better appeals to consumers and their assumed changes in priorities. I argue Foundayo is better suited for this shift than Oral Wegovy and will ultimately cannibalize much of the stellar performance the latter drug has seen thus far for one obvious but critical key reason: dietary restrictions.

Fasting Restriction Explanation

To be specific, the difference in dietary restrictions stems from each drug's molecular structure, with Oral Wegovy having a small peptide structure and Foundayo having a small molecule structure. With a peptide structure, the patient's gut cannot tell the difference between the pill and a piece of food, exposing it to digestive enzymes that destroy most of it before the active ingredient reaches the bloodstream. To compensate for this, Oral Wegovy includes an absorption-enhancing ingredient, Salcaprozate Sodium (SNAC), that temporarily makes the stomach lining more permeable and allows a portion of the peptide to pass through.³² However, if the pill is taken with food, drinks, or excess amounts of water, the absorption and efficacy of the pill declines significantly.³³ On the other hand, Foundayo's small molecule structure is more compact and tightly bonded, allowing it to better resist stomach acid and reach the bloodstream without a protective shielding ingredient like SNAC. What this difference ultimately results in is one simple but arguably paramount difference between the two drugs: Oral Wegovy must be

³² Caroline Twarog, Sabrina Fattah, John Heade, Sam Maher, Elias Fattal, and David J. Brayden, "Intestinal Permeation Enhancers for Oral Delivery of Macromolecules: A Comparison between Salcaprozate Sodium (SNAC) and Sodium Caprate (C10)," *Pharmaceutics* 11, no. 2 (2019): 78, <https://pmc.ncbi.nlm.nih.gov/articles/PMC6410172/>.

³³ Tine A. Bækdal, Astrid Breitschaft, Morten Donsmark, Stine J. Maarbjerg, Flemming L. Søndergaard, and Jeanett Borregaard, "Effect of Various Dosing Conditions on the Pharmacokinetics of Oral Semaglutide, a Human Glucagon-Like Peptide-1 Analogue in a Tablet Formulation," *Diabetes Therapy* 12, no. 7 (July 2021): 1915–1927, <https://pmc.ncbi.nlm.nih.gov/articles/PMC8266944/>.

taken first thing after waking with up to four ounces of water and must be followed by a 30-minute fast post-administration whereas Foundayo can be taken at any point in the day without fasting.

This difference may seem insignificant, and Novo Nordisk's CEO Mike Doustard reinforced this view during the 2026 J.P. Morgan Healthcare conference, citing the 1.5 million patients on Rybelsus as evidence of that.³⁴ However, the demand for oral semaglutide approved for T2D and oral semaglutide approved for weight loss face a structural difference in that the former has no direct competitor currently. As it remains the only oral T2D medication on the market, extrapolating Rybelsus trends in consumer demand misrepresents the current state of the oral weight-loss market, where Foundayo is available and yet to be fully scaled. Furthermore, as Foundayo looks to additionally obtain T2D approval, the singular oral pill could serve as a combination weight loss and T2D drug that provides patients with both benefits rather than a single indication, as is the case with Rybelsus and Oral Wegovy.³⁵ Finally, the reason that Foundayo TRx currently remains low following its launch is not a sign of weak consumer demand, but instead a strategically slow drug rollout on Lilly's part.

The reasoning for this delayed rollout is due to Foundayo being a new molecule, new modality, and new brand compared to oral semaglutide's familiar molecule and brand. Given that the pill was also entering a market dominated exclusively by Oral Wegovy, which had a three-month lead over Foundayo, management decided to take a three-gate sequence for the drug's launch in the following order: educating health care providers on the product, building payer

³⁴ GlobalData Healthcare, "JPM26: Novo Nordisk Reshapes Its Strategy under New CEO," *Yahoo Finance*, January 14, 2026, <https://finance.yahoo.com/news/jpm26-novo-nordisk-reshapes-strategy-041306688.html>.

³⁵ Eli Lilly and Company, "Lilly's Oral GLP-1 Foundayo (Orforglipron) Delivered Superior A1C Control and Weight Loss in Three Pivotal Type 2 Diabetes Trials," *News Release*, June 8, 2026, <https://investor.lilly.com/news-releases/news-release-details/lillys-oral-glp-1-foundayo-orphorglipron-delivered-superior-a1c>.

access, and ending with building consumer awareness.³⁶ Throughout Quarter 2 2026, the company focused on developing the first two steps, signing three major pharmacy benefit managers, growing prescribers sixfold from April to August, and gaining increased exposure through the Medicare GLP-1 Bridge program, which allowed eligible patients to receive the drug for \$50 a month out of pocket through December 2027.³⁶ Direct-to-consumer marketing to raise consumer awareness has just begun rolling out for the oral drug as well, meaning that the full results of whether Foundayo will overtake Wegovy will materialize in Q3 or Q4 of 2026.

To conclude, my thesis for Foundayo overtaking Oral Wegovy as the leading oral weight-loss GLP-1 relies on the consumer rationale and data-backed assumptions regarding the change in consumer GLP-1 preferences paired with the start of a full-fledged marketing effort behind Foundayo to raise consumer awareness of the drug. I argue that the priced-in reliance on Oral Wegovy's continued TRx performance will fail to materialize as a continued slowdown and loss of Novo's ~90% US oral obesity market share develops by the end of 2026, limiting yet another core revenue driver for the company and offering strong financial upside by assuming a short position on NVO.

Principal Risks and Counterarguments

Given that this position ultimately depends on the short-term development of the company over the next two quarters rather than long-term financial viability of Novo Nordisk, I

³⁶ Eli Lilly and Company, "Q1 2026 Earnings Call," *Eli Lilly and Company Investor Relations*, April 30, 2026, <https://investor.lilly.com/events/event-details/q1-2026-earnings-call>; Eli Lilly and Company, "Q2 2026 Earnings Call," *Eli Lilly and Company Investor Relations*, August 5, 2026, <https://investor.lilly.com/events/event-details/q2-2026-earnings-call>.

argue the current pipeline portfolio and semaglutide LOE in various countries are largely insignificant in terms of its impact on NVO's share price in the remaining two quarters. Furthermore, given that the stock has historically had significant share price rises and drops surrounding clinical trial efficacy readouts, it's notable that there remain no key efficacy readouts for Novo Nordisk's drugs planned for Q3 or Q4 of 2026. While this lack of clinical trial readouts eliminates the possibility of internal trial-induced volatility in NVO's share price, there remain multiple key scenarios that could cause the stock to see strong upside or downside potential.

Risk 1: Foundayo Restrictions Alienate Consumers

While I have outlined the lack of dietary restrictions with Foundayo as a core component of consumers favoring the drug over Oral Wegovy, that is not to say there are no restrictions related to Foundayo. As noted in the clinical trial efficacy table in this report's appendix, Foundayo has multiple restrictions that make initially starting the drug slightly tedious. The first restriction with Foundayo involves patients taking medications that slow down or block the CYP3A4 liver enzyme, like the HIV drug ritonavir or fungal infection drug ketoconazole.³⁷ Patients who take these drugs that lead to lower levels of the CYP3A4 enzyme have a harder time breaking down Foundayo once administered, leading to a buildup of Foundayo in the bloodstream instead of it being cleared out normally. This means that the presence of CYP3A4 inhibitors can increase exposure to Foundayo beyond a safe amount, leading to an increased risk

³⁷ Dresser GK, Spence JD, Bailey DG. Pharmacokinetic-pharmacodynamic consequences and clinical relevance of cytochrome P450 3A4 inhibition. Clin Pharmacokinet. 2000 Jan;38(1):41-57, <https://pubmed.ncbi.nlm.nih.gov/10668858/>.

of associated adverse reactions from the GLP-1 drug and reduced efficacy.³⁸ While the number of people who use CYP3A4 inhibitors like ritonavir specifically between December 2021 and August 2023 hovered close to 2 million, the limitation on titrating up to the max dose for Foundayo would, at worst, only target 2% of the total obese population in the US, and likely wouldn't have major short-term impacts on TRx scaling.³⁹

The second key restriction associated with Foundayo involves people taking simvastatin, a prescription statin medication used to lower LDL cholesterol and triglycerides while raising beneficial HDL cholesterol to lower the risk of MACE. Rather than how CYP3A4 inhibitors can negatively affect the amount of Foundayo in your bloodstream, Foundayo itself affects simvastatin by increasing exposure to simvastatin acid levels almost two-fold when the two drugs are coadministered. At high enough doses of simvastatin, acid levels can raise the risk of muscle toxicity and cause muscle-impairing diseases including myopathy and, in severe cases, the muscle tissue destroying condition rhabdomyolysis. As a result, people using simvastatin are capped at 20 mg (half of the common maximum dose) per day, providing a chance for further consumer TAM limitation among the cohort who uses the MACE reduction drug currently.⁴⁰ By using data from 2014 simvastatin-specific use and applying that percentage to the number of adult Americans who meet the criteria to take cholesterol-lowering statins currently, I estimate

³⁸ U.S. Food and Drug Administration, "Foundayo (orforglipron) Tablets, for Oral Use: Prescribing Information," revised April 2026, https://www.accessdata.fda.gov/drugsatfda_docs/label/2026/220934Orig1s000lbl.pdf.

³⁹ Abby E. Rudolph et al., "Proportion of Patients in the United States Who Fill Their Nirmatrelvir/Ritonavir Prescriptions," *Open Forum Infectious Diseases* 11, no. 8 (2024), <https://pmc.ncbi.nlm.nih.gov/articles/PMC11343940/>;

Lydia M. M. Vermeer et al., "Evaluation of Ketoconazole and Its Alternative Clinical CYP3A4/5 Inhibitors as Inhibitors of Drug Transporters: The In Vitro Effects of Ketoconazole, Ritonavir, Clarithromycin, and Itraconazole on 13 Clinically-Relevant Drug Transporters," *Drug Metabolism and Disposition* 44, no. 2 (2016): 244–249, <https://doi.org/10.1124/dmd.115.067744>.

⁴⁰ "Simvastatin Dosage Guide + Max Dose, Adjustments," Drugs.com, last updated July 2, 2026, <https://www.drugs.com/dosage/simvastatin.html>.

there exist roughly 36.75 million patients who would have the potential to be impacted by this restriction.⁴¹

While that number is significantly higher than those who would be potentially affected by the CYP3A4 inhibitor restriction, it's important to note that this calculation includes the total number of Americans who qualify for taking statins rather than those who currently adhere to that qualification. Furthermore, the FDA restriction only applies to Simvastatin specifically, not the entire MACE risk reduction statin class. In Foundayo's United States Prescribing Information (USPI) PDF, which outlines all the key data related to FDA-approved drugs like Foundayo, tests found that combining other statins with Foundayo increases exposure to statin acid levels.⁴² However, key statins including atorvastatin and rosuvastatin had smaller increases in exposure when combined with Foundayo than simvastatin did, increasing by 1.5-fold and 1.7-fold, respectively.⁴² As a result, the FDA did not issue a dosing limit on these other statins, meaning consumers who were drawn to Foundayo but were taking simvastatin could switch relatively easily to other non-restricted statins that have similar MACE reduction effects.⁴³

The final, and arguably most significant, restriction with Foundayo is a strong recommendation for patients using oral hormonal contraceptives to switch to a non-oral

⁴¹ Qiuping Gu et al., "Prescription Cholesterol-Lowering Medication Use in Adults Aged 40 and Over: United States, 2003–2012," NCHS Data Brief no. 177 (Hyattsville, MD: National Center for Health Statistics, December 2014), <https://www.cdc.gov/nchs/products/databriefs/db177.htm>;

Kara Gavin and Elaine Vitone, "More Than Half of U.S. Adults Now Qualify for Statins Under New Guidelines," Michigan Medicine Health Lab, July 21, 2026, <https://www.michiganmedicine.org/health-lab/more-half-us-adults-now-qualify-statins-under-new-guidelines>.

⁴² Eli Lilly and Company, "Highlights of Prescribing Information: FOUNDAYO (orforglipron) tablets, for oral use," revised July 2026, <https://pi.lilly.com/us/foundayo-uspi.pdf>

⁴³ Furio Colivicchi, Michele Massimo Gulizia, Laura Franzini, Giuseppe Imperoli, Lorenzo Castello, Alessandro Aiello, Claudio Ripellino, and Nazarena Cataldo, "Clinical Implications of Switching Lipid Lowering Treatment from Rosuvastatin to Other Agents in Primary Care," *Advances in Therapy* 33, no. 11 (November 2016): 2049–2058, <https://pmc.ncbi.nlm.nih.gov/articles/PMC5083763/>;

James C. Moon and Richard G. Bogle, "Switching Statins: Using Generic Simvastatin as First Line Could Save £2bn Over Five Years in England," *BMJ* 332, no. 7554 (2006): 1344, <https://pmc.ncbi.nlm.nih.gov/articles/PMC1476712/>.

contraceptive method or add barrier contraception for 30 days after initiation and each dose titration of the oral GLP-1. Given that Foundayo and many GLP-1s slow down stomach digestion, taking the drug can reduce the absorption amount of an oral hormonal contraception. To remain conservative, the FDA recommendation aims to limit the possibility of under absorption of oral hormonal contraceptives. Given that an estimated 151 million women worldwide and roughly 9 million women in the US of reproductive age use oral hormonal contraceptives, the recommendation holds significant merit in limiting a key cohort from using Foundayo.⁴⁴ However, the risk itself is slightly mitigated by the fact that the restriction is in fact a recommendation, and more accurately a generic GLP-1 recommendation rather than a Foundayo-specific one.

When comparing the two USPI sheets for Foundayo and its counterpart Oral Wegovy, a noticeable difference exists in that Oral Wegovy, a digestion-slowing GLP-1 like Foundayo and every other major injectable covered previously, doesn't have the FDA oral hormonal contraceptive switch recommendation.⁴⁵ The reason for this difference is not directly due to Oral Wegovy being superior to Foundayo and providing less risk of accidental pregnancies for patients using oral hormonal contraceptives, although the effect of its unconventional SNAC-

⁴⁴ Therese Johansson, Sara Vinther Larsen, Minh Bui, Weronica E. Ek, Torgny Karlsson, and Åsa Johansson, "Population-Based Cohort Study of Oral Contraceptive Use and Risk of Depression," *Epidemiology and Psychiatric Sciences* 32 (2023): e39, <https://pmc.ncbi.nlm.nih.gov/articles/PMC10294242/>; Kimberly Daniels and Joyce C. Abma, "Current Contraceptive Status Among Females Ages 15–49: United States, 2022–2023," NCHS Data Brief no. 539 (Hyattsville, MD: National Center for Health Statistics, August 2025), <https://www.cdc.gov/nchs/products/databriefs/db539.htm>; World Health Organization, "Women of Reproductive Age (15–49 Years) Population (Thousands)," Maternal, Newborn, Child and Adolescent Health and Ageing Data Portal, accessed August 12, 2026, [https://platform.who.int/data/maternal-newborn-child-adolescent-ageing/indicator-explorer-new/mca/women-of-reproductive-age-\(15-49-years\)-population-\(thousands\)](https://platform.who.int/data/maternal-newborn-child-adolescent-ageing/indicator-explorer-new/mca/women-of-reproductive-age-(15-49-years)-population-(thousands)).

⁴⁵ Eli Lilly and Company, "Highlights of Prescribing Information: FOUNDAYO (orforglipron) tablets, for oral use," revised July 2026, <https://pi.lilly.com/us/foundayo-uspi.pdf>; U.S. Food and Drug Administration, "Highlights of Prescribing Information: WEGOVY (semaglutide) injection, for subcutaneous use; WEGOVY (semaglutide) tablets, for oral use," revised January 2026, https://www.accessdata.fda.gov/drugsatfda_docs/label/2026/218316s002lbl.pdf

based absorption compared to Foundayo's traditional GI tract absorption on the degree to which oral hormonal contraceptives are absorbed is still untested. Instead, Novo Nordisk ran a dedicated study that paired Oral Wegovy against oral hormonal contraceptives and found no clinically significant differences in effect of either drug while Eli Lilly simply didn't run a dedicated test showing Foundayo's effect on the oral contraceptives. As a result, the FDA eliminated the recommendation to switch from Oral Wegovy's USPI and applied the generic recommendation to Foundayo's USPI. Interestingly, however, when comparing SC Wegovy's USPI and Zepbound's USPI, no recommendation to switch contraceptive methods exists for the former while one does exist for the latter.⁴⁶ The reason for this is instead reversed however, as a clinical trial for tirzepatide found it can reduce oral hormonal contraceptive efficacy due to delayed gastric emptying while no related trial is listed at all for SC Wegovy. The reason for this lack of a recommendation may be due to both SC Wegovy and oral Wegovy using the same active ingredient, semaglutide, but there remains a lack of a cited trial showing SC Wegovy's effect on oral hormonal contraceptive absorption. Nevertheless, the fact that Foundayo ultimately does have the FDA recommendation to switch while Oral Wegovy does not poses the most notable possibility of consumer alienation towards Foundayo and should be considered as a risk to TRx uptake as patients choose Oral Wegovy over Foundayo in the short term. After all, the central thesis of Oral Wegovy ceding its market dominance to Foundayo relies on consumers prioritizing convenience above all else, and being strongly recommended to switch from one's

⁴⁶ U.S. Food and Drug Administration, "Highlights of Prescribing Information: WEGOVY (semaglutide) injection, for subcutaneous use; WEGOVY (semaglutide) tablets, for oral use," revised January 2026, https://www.accessdata.fda.gov/drugsatfda_docs/label/2026/218316s002lbl.pdf; Eli Lilly and Company, "Highlights of Prescribing Information: ZEPBOUND (tirzepatide) injection, for subcutaneous use," revised April 2026, <https://pi.lilly.com/us/zepbound-uspi.pdf>.

own oral contraceptive method to a barrier or injection contraceptive method is anything but convenient.

Risk 2: Volatility-Prone FDA Approvals and Trial Readouts

As seen with the ~30% and ~9% drop in NVO's share price following the negative CagriSema REDEFINE-4 and Ziltivekimab ZEUS clinical trial efficacy readouts, respectively, investors have historically reacted strongly to clinical trial data, and understandably so. The failure to achieve each drug's primary end points of showing non-inferiority to tirzepatide for CagriSema and reducing MACE compared to a placebo for Ziltivekimab severely limited Novo Nordisk's ability to reclaim parts of its once-dominant GLP-1 weight-loss market share or to add on a new \$3-\$5 billion arm of the company. Given this, clinical trial results readouts exist as major points of share price volatility and, due to the difficulty of predicting their outcomes, uncertainty. One can see this difficulty given that multiple reports on Novo Nordisk following up to its release of ZEUS results leaned towards Ziltivekimab showing positive MACE reduction top-line results, especially based on using prior trial data from Canakinumab's CANTOS trial as a comparable. Given the limitation to data and general difficulty of predicting trial outcomes in the first place, I have foregone attempting to include any future 2026 trial readouts within this report and my valuation.

However, there remain four key points which could provide NVO share price appreciation if positive results are released: the US FDA decision for CagriSema's obesity indication, the FDA decision for 25 mg T2D oral semaglutide (Rybelsus/Oral Ozempic), Phase 3 safety results for Efruxifermin in F1-F4 fibrosis, and the US and EU decision for Novo's

Hemophilia A drug [Denecimig](#). Given that the last event is positioned within Q3 2026 in Novo Nordisk's Q2 2026 Investor presentation while the CagriSema approval, oral semaglutide 25 mg decision, and Efruxifermin safety results are positioned within Q4 2026, the Denecimig decision will likely be the first key volatility point for the stock. While there aren't many true mitigants to pricing in these readouts and decisions that I believe I could accurately propose using trial comparables or historical FDA decision criteria, I instead see potential mitigants in the specific type of event that each of these volatility points represent.

While the major efficacy-focused clinical trials I listed previously had significant effects on NVO's share price depreciation, events like regulatory approvals and safety readouts have historically had more muted share price impacts. In the days following Oral Wegovy's FDA approval for chronic weight management on December 22, 2025, NVO's share price saw only a 7% appreciation. Similarly, in the week following the FDA approval of Subcutaneous Wegovy 2.4 mg and its high dose 7.2 mg dose, the share price only appreciated 4% and depreciated by about 2%, respectively. In terms of safety readouts, the Ziltivekimab ZEUS readout did additionally include safety details, which showed side effects and serious adverse event rates in line with the placebo group, but as with the vast majority of trials the share price action was largely driven by the primary endpoint efficacy miss.

While there haven't been many clinical trials focused on safety as a primary endpoint like Efruxifermin's upcoming SYNCHRONY Real-World trial, some other comparable trials by Novo Nordisk where safety was a core focus include the SELECT SC Wegovy trial, FLOW Ozempic trial, and SOUL Rybelsus trial. In these scenarios, NVO saw a roughly 17% increase, 2% decrease, and 3% decrease, respectively. While it's difficult to isolate how much of the price action was driven by the safety readouts specifically, major jumps like those seen in the SELECT

SC Wegovy trial were almost entirely due to the reinforcement of Wegovy as a possible MACE treatment. Due to this, a trial like SYNCHRONY Real-World, where the primary endpoint is the safety and tolerability of the drug, is largely unprecedented and therefore remains arguably the most important event of the four listed for unprecedented upwards price action following a theoretical positive readout. Given that the two Wegovy FDA approval events only had low single-digit price appreciation and even price depreciation, I believe the approvals of Denecimig, CagriSema, and 25 mg Rybelsus are largely already priced in. Furthermore, while the share price appreciation surrounding Oral Wegovy showed notably high single-digit increases in NVO's value, I argue that was largely a factor of Oral Wegovy being able to subsequently take advantage of the first-mover advantage over Foundayo rather than simply due to its approval.

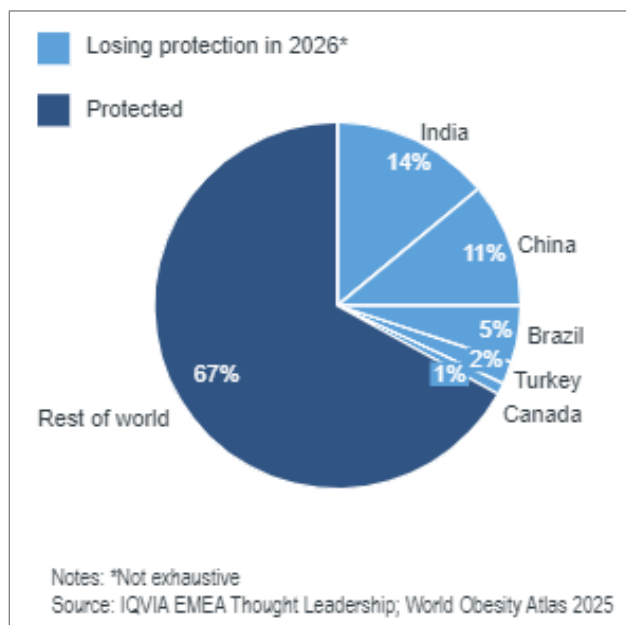
Risk 3: Semaglutide LOE Demonstrates Strong Brand-Name Retention

The final and potentially most threatening risk that could cause NVO to see strong price appreciation is the continued TRx development of semaglutide-based products in countries where patents for the active ingredient are expiring or already expired. Several countries have already seen the semaglutide LOE go into effect and generics hit the market including India, China, Canada, Brazil, and Turkey, which alone make up 40% of the world's population and an estimated 33% of the world's obese population, as shown in Figure 4.1.⁴⁷ While investors have largely already priced in the expected patient retention and potential TRx increases following price decreases for the brand name drugs, what's uncertain and largely unprecedented is the

⁴⁷ Aurelio Arias and Sarah Rickwood, "Off-Patent Semaglutide in 2026: The next Revolution in Anti-Obesity Medications," IQVIA, August 5, 2025, <https://www.iqvia.com/locations/emea/blogs/2025/07/off-patent-semaglutide>.

degree to which the traditionally inelastic demand for GLP-1 drugs will react to plummeting list and net prices.

Figure 4.1 – Global Obese Population Experiencing Semaglutide 2026 LOE:



(A graph showing the current percentage of adults living with obesity in the world. Key markets in which semaglutide patents expire this year and the percentage of the global obese population they represent are shown in light blue.)

While the access I have to non-US TRx trends is limited, Figure 4.1 shows the portion of current adults living with obesity worldwide and highlights key countries where semaglutide is facing 2026 LOE. Since these patent expirations and an annual stepdown in top-line contribution from semaglutide-based drugs is likely already priced in, a surge in brand-name semaglutide drugs like Ozempic, which has arguably the strongest global brand name recognition of any drug in history, SC Wegovy, and the two oral formulations for weight loss and T2D, Oral Wegovy and Rybelsus, could be a strong catalyst for increasing the future top-line contributions that the decade-old brand can provide to Novo before it ultimately loses out to generic competition.

High-level Valuation Sensitivities

For my DCF, I use a 10-year projection period to include my case-dependent perspective regarding the impact of the US and future countries' semaglutide LOE up to 2035. However, I forgo assuming the TRx trends and top-line contributions of current pipeline drugs like CagriSema, Denecimig, or Ziltivekimab due to my lack of defensible foresight of those drugs' future performance as the GLP-1 market matures. To compensate for this significant understatement in FCF contributions for current pipeline drugs that will become commercially available in the next ten years, I apply a 3% terminal growth rate (2% above consensus) which sits above my reverse DCF's implied 10-year revenue growth rate of 0.27%.

Figure 5.1 - Ten-Year Reverse DCF:

Modifiable Long-Term Assumptions										
Assumption	Value									
Tax Rate	21.00%									
WACC	6.79%									
Terminal Growth	1.00%									
FY Assumptions										
10-Year Projection										
Fiscal Period	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E
Revenue Growth	0.27%	0.27%	0.27%	0.27%	0.27%	0.27%	0.27%	0.27%	0.27%	0.27%
EBIT Margin	41.6%	40.2%	40.4%	40.2%	40.1%	39.9%	39.8%	39.6%	39.5%	39.3%
D&A/Rev	6.5%	7.1%	6.9%	6.8%	6.8%	6.7%	6.7%	6.6%	6.6%	6.5%
CapEx/Rev	19.6%	11.0%	9.5%	9.3%	9.1%	8.9%	8.7%	8.5%	8.3%	8.1%
Change in WC/Rev	-21.4%	-21.4%	-21.4%	-21.4%	-21.4%	-21.4%	-21.4%	-21.4%	-21.4%	-21.4%
DCF Model										
10-Year Projection										
Fiscal Period	2026E	2027E	2028E	2029E	2030E	2031E	2032E	2033E	2034E	2035E
Revenue	309883	310704	311528	312354	313181	314011	314844	315678	316515	317354
EBIT	129025	125024	125786	125651	125514	125376	125236	125094	124951	124806
NOPAT	101930	98769	99371	99264	99156	99047	98936	98824	98711	98597
D&A	20155	21967	21408	21308	21208	21107	21006	20904	20801	20697
CapEx	60747	34178	29595	29049	28499	27947	27391	26832	26270	25705
Change in WC	-175	-176	-176	-177	-177	-178	-178	-179	-179	-180
FCF	61513	86734	91360	91701	92042	92385	92729	93074	93421	93768
NPV of FCF	57602	76056	75018	70510	66273	62290	58547	55028	51721	48613
Implied Share Price Calculation										
	10-Year									
Sum of NPV FCF	621657									
Terminal Value	1635683.4									
NPV of Terminal Value	847995.18									
EV	1469652									
Cash	21626									
Debt	146382									
Minority Interest	0									
Equity Value	1344896									
WASO	4429									
Implied Share Price	303.65									
Current Share Price	303.65 kr									
Implied Revenue Growth Solver										
	10-Year									
Revenue Base	309064									
Implied Revenue CAGR	0.27%									
Terminal Growth Rate	WACC									
		6.5%	7.0%	6.8%	8.0%	8.5%				
	-1.0%	1.82%	2.67%	2.32%	4.28%	5.06%				
	0.0%	0.84%	1.76%	1.38%	3.48%	4.31%				
	1.0%	-0.33%	0.68%	0.27%	2.56%	3.45%				
	2.0%	-1.75%	-0.61%	-1.08%	1.48%	2.44%				
	3.0%	-3.56%	-2.21%	-2.76%	0.17%	1.25%				

(A condensed snapshot of my reverse DCF, highlighting the 10-year implied revenue growth CAGR and all key inputs associated with it. The sensitivity table outlines 10-year revenue CAGRs given different inputs for the WACC and Terminal Rate.)

As shown in Figure 5.1, the 10-year implied growth rate from my reverse DCF is derived from inputting the share price of Novo Nordisk's Danish stock, NOVO B, as of Friday, August 7th, 2026, in addition to the following assumptions: operating margins compress from 41.6% in 2026 to 39.3% in 2035, Depreciation and Amortization hovers near 7% of revenue (steady state levels), CapEx decreases as a percent of revenue from an expected 19.6% in 2026 due to the

Kalundborg Wegovy manufacturing buildout to 8.1% in 2035 (towards steady state levels), and change in working capital as a percentage of change in revenue remains flatlined at -21.4% based on a quarterly average from Q1 2020 to Q1 2026. The revenue base to which I apply all my assumptions is Novo Nordisk's FY 2025. Additionally, to exclude FX noise and keep everything easily comparable, I conduct my entire model in millions of Danish Kroner and convert the final implied share price for each case to USD at the end using current exchange rates of \$0.15 per Danish Krone. As the NVO ADR closely tracks the Danish stock NOVO B, typically within low single-digit Danish Kroner or within a \$2 range, I use the Danish stock NOVO B's share price as the input for my valuation percent spreads from implied share prices and reverse DCF inputs.

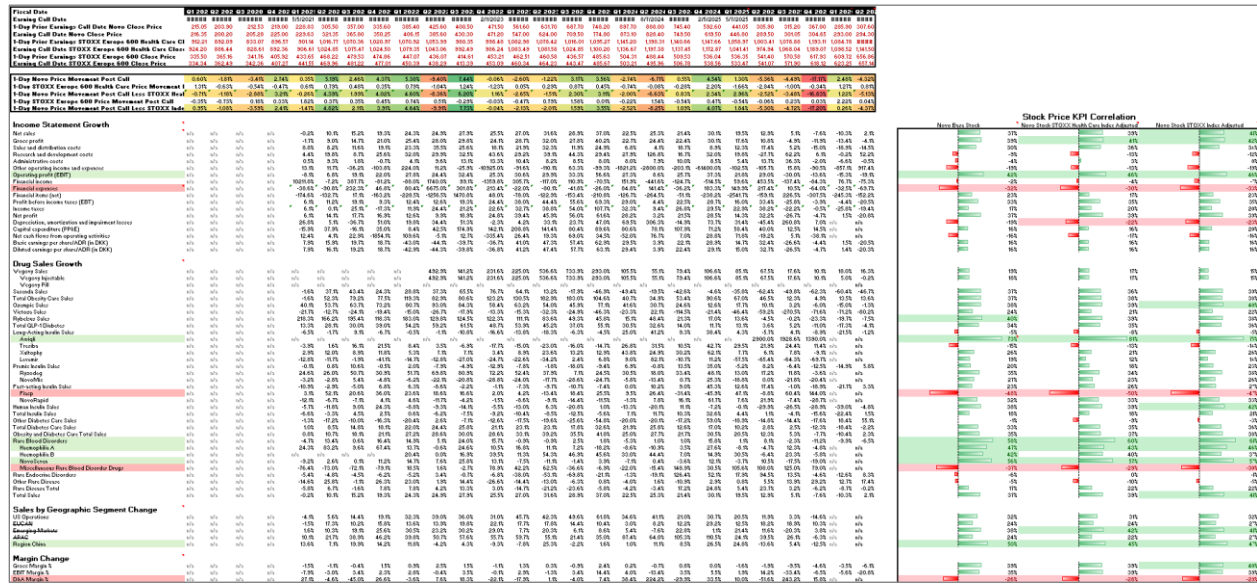
Figure 5.2 – Calculated WACC

Inputs		Aswath Damodaran: Credit Rating Spread Table				Aswath Damodaran: Betas by Sector (Global)		
		Interest Coverage Ratio		Rating	Spread	Industry Name	Number of firms	Beta
Risk-free Rate	2.80%	-100000	0.199	D	19.00%	Advertising	419	1.27
Equity Risk Premium	4.23%	0.2	0.649	C	16.00%	Aerospace/Defense	319	1.23
Country Risk Premium	0.00%	0.63	0.799	CC	12.81%	Air Transport	130	0.99
Effective Tax Rate	16.75%	0.8	1.249	CCC	8.85%	Apparel	1207	0.76
Share Price (Friday, August 7, 2026)	303.65 kr	1.25	1.499	B-	5.09%	Auto & Truck	165	1.41
Shares Outstanding (million)	4429.1	1.5	1.749	B	3.21%	Auto Parts	797	1.45
Outputs		1.75	1.999	B+	2.75%	Bank (Money Center)	604	0.7
Novo's Levered Beta	0.83	2	2.249	BB	1.84%	Banks (Regional)	825	0.51
Average Relevered Beta	0.79	2.25	2.499	BB+	1.38%	Beverage (Alcoholic)	217	0.79
Self-Calculated WACC	6.79%	3	2.999	BBB	1.11%	Beverage (Soft)	94	0.99
		4.25	3.499	A-	0.89%	Broadcasting	127	0.83
		5.5	4.499	A	0.78%	Brokerage & Investment Banking	633	0.98
		6.5	4.999	AA	0.70%	Building Materials	469	1
		8.5	5.499	AAA	0.55%	Business & Consumer Services	994	0.99
					0.40%	Cable TV	43	1.1
						Chemical (Basic)	909	1.26
						Chemical (Diversified)	63	1.17
						Chemical (Specialty)	952	1.15
						Coal & Related Energy	212	1.14
						Computer Services	1235	1.12
						Computers/Peripherals	343	1.49
						Construction Supplies	804	1.03
						Diversified	329	0.8
						Drugs (Biotechnology)	1193	1.23
						Drugs (Pharmaceutical)	1260	1.06
						Education	281	0.81
						Electrical Equipment	1158	1.37
						Electronics (Consumer & Office)	122	1.2
						Electronics (General)	1481	1.6
						Engineering/Construction	1390	1
						Entertainment	733	1.01
						Environmental & Waste Services	397	1.1
						Farm/Agiculture	428	0.67
						Financial Svcs. (Non-bank & Insura	1138	0.82
						Food Processing	1450	0.67
						Food Wholesalers	183	0.68
						Furn/Home Furnishings	383	0.97
						Green & Renewable Energy	258	0.84
						Healthcare Products	847	1.18
						Healthcare Support Services	478	0.92
						Healthcare Information and Technol	430	1.29
						Homebuilding	160	0.96
						Hospitals/Healthcare Facilities	249	0.76
						Hotel/Gaming	656	0.8
						Household Products	588	0.83
						Information Services	86	1.02
						Insurance (General)	204	0.51
						Insurance (Life)	143	0.8
						Insurance (Prop/Cas.)	248	0.43
						Investments & Asset Management	1315	0.75
						Machinery	1553	1.37
						Metals & Mining	1874	1.24
						Office Equipment & Services	139	0.76
						Oil/Gas (Integrated)	34	0.69

(A snapshot of the inputs and outputs for my three WACC calculation methods.)

While all the assumptions for EBIT margins, D&A, CapEx, and other inputs were primarily the mean of consensus numbers pulled from NVO's LSEG stock reports, one of the most intriguing inputs to determine was the Weighted Average Cost of Capital. Figure 5.2 shows the methods I used for calculating Novo's WACC through three different unlevering and relevering methods, including a peer beta and Novo debt-to-equity ratio, a peer beta and peer debt-to-equity ratio, and a global pharma beta and Novo debt-to-equity ratio. In all three of these cases, my WACC ended in the low 5% to high 6% range, but never once broke into 7%, while the three reports I sourced that outlined their WACC all listed it as above 7%, with a mean of 7.5%. Considering I was unable to determine how analysts derived their WACCs, I opted to use my self-calculated 6.79% WACC average for the reverse derived from my WACC build. However, I included a terminal growth rate and WACC sensitivity table for implied annual top-line CAGR in Figure 5.1 to the implied annual sales CAGRs for consensus 7.5% WACC and 1% terminal rate.

Figure 5.3 – KPI Backtest:



(A snapshot of a KPI Backtest used to analyze the most prominent positively and negatively correlated items to earnings call day share price action.)

Furthermore, there are multiple insights that I derived from the KPI Backtest I conducted to analyze the effect on earnings call day's share price action for specific metrics and drug groups' growth or slumps. The test uses a year-over-year percent change comparison to adjust for seasonality and shows the impact of every major income statement metric, drug cohort and drug brand, geographic segment performance, and margin type. Unsurprisingly, as shown in Figure 5.3, operating margin, Total Obesity GLP-1 Sales, and Total Diabetes GLP-1 Sales all saw strong, positive correlation with NOVO B's share price action across normal and index-adjusted correlation measures. On the other hand, financial expenses and D&A margin were the main negatively correlated share price action inducers, with negative share price earnings call day reactions being notably correlated with those two measures increasing.

While there are not enough quarters of data to measure Oral Wegovy on even a seasonality-adjusted YoY basis, its T2D-approved form Rybelsus shows strong positive correlation with NOVO B's share price change over the past 26 quarters and could possibly serve as a proxy for its weight-loss counterpart. This trend also aligns with the sharp decrease following Novo's recent Q2 2026 results, which showed softer Oral Wegovy sales as TRx began to see declines in the strong growth rate it exhibited initially, as shown in Figure 3.1.

Furthermore, there were multiple stand-out items that had extremely strong positive and negative correlations that one wouldn't expect, including the Fast-Acting insulin Fiasp, the Long-Acting insulin Awiqli, specific rare disease categories, and the Region China geographical region. While the Awiqli correlation is explainable through its only 3 quarters of data, the negative correlation of Fiasp and Miscellaneous Rare Blood Disorder Drugs but positive correlation of the Rare Blood Disorder segment overall and Region China geographic segment is more complex. The strong positive correlation for the Rare Blood Disorder category overall, as well as Hemophilia A and NovoSeven drugs, is likely viewed as a key non-GLP-1 diversifier segment that holds prominence as Novo continues to bring innovations to the space, seen most recently with the soon-to-be approved Denecimig. The Miscellaneous Rare Blood Disorder Drugs segment is likely just a model error rather than an actual sign of correlation, considering I used that line item as the plug for all of the remaining income for the segment overall that was not attributable to the major line-item rare disease drugs themselves.

Region China presents the most intriguing readout of them all, specifically because of the reclassification I embedded within my Revenue build following Novo Nordisk's Q1 2025 listing of new geographical reporting segments. Instead of rebuilding out a new segment of the revenue build with the specific new geographical units from 2025 onwards, I embedded the most similar

markets before and after the reclassification together. While legacy EMEA (Europe, Middle East, Africa) was reclassified into EUCAN (Europe and Canada), legacy Rest of World was reclassified into Emerging Markets, and legacy North America Less US (Canada) was reclassified into APAC (Asia-Pacific countries, a poor reclassification), US Operations and Region China remained the two geographical segments that were unchanged region wise before and after Q1 2025. Therefore, Region China shows over 10% higher positive correlation in each of the three measures compared to the US, suggesting that the market was likely a key expansion opportunity, outlined by Figure 4.1, in investors' minds before the 2031 US semaglutide LOE and rise of generic US competitors.

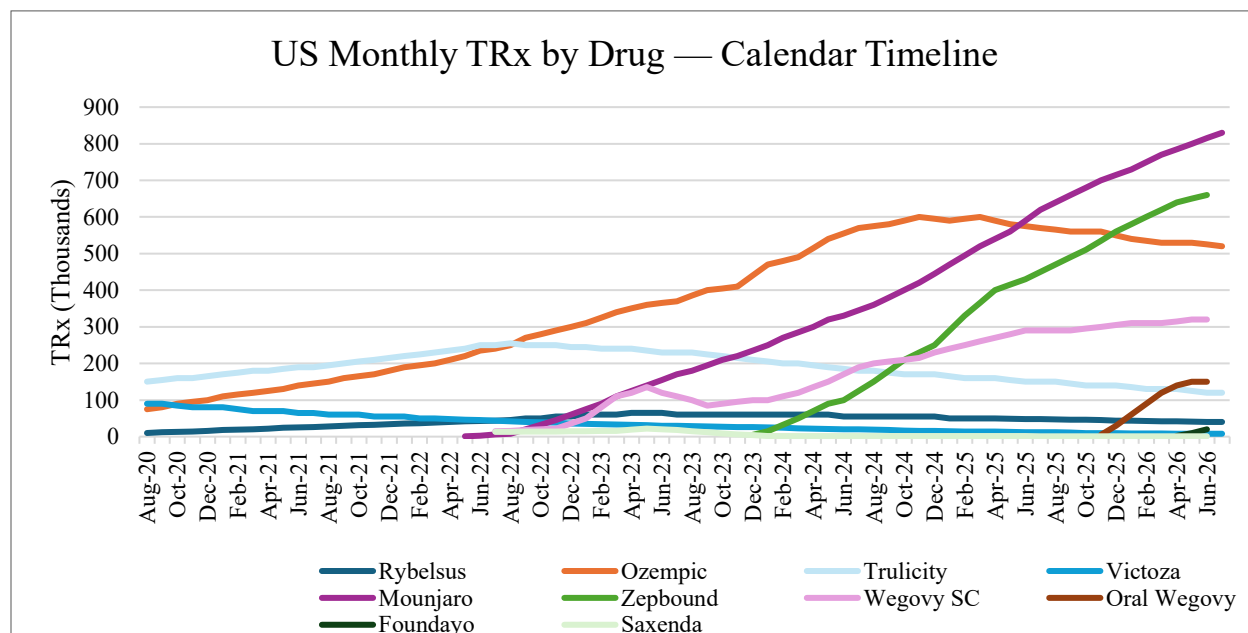
Three Case Valuation

Base Case

My share price valuation of NVO revolves around a base, upside, and downside case, each with its own assumptions and implied share prices derived from a ten-year projection period DCF. The cases and their respective assumptions are outlined below, beginning with the base case that represents the most likely outcomes for each line item on the revenue build given current trends and available information. For the base case, I assume largely no change in current operations and TRx trends, extrapolating current and historical market performance forward. In terms of Oral Wegovy, I assign a 2% top-line TRx growth rate per week for the rest of 2026 based on the recent TRx slowdown for Oral Wegovy to around 2.2%, reflecting the partial realization of my thesis and a dual growth scenario where Oral Wegovy maintains moderate growth even as Foundayo scales. For the rest of the projection period through 2031, I flatline a 1.5% growth rate annually in top-line sales for Oral Wegovy given the TRx trends observed

between SC Wegovy following the FDA approval and commercial launch of Zepbound and assuming increasing non-US semaglutide generics provide headwinds to brand name Oral weight-loss semaglutide's growth. From 2032 to 2035, I model the US semaglutide LOE's effect on Oral Wegovy using a generic-entry sales-retention framework derived from a study of 140 originator drugs approved in the US between 2010 and 2019.²⁴ Using that framework, I assign a 73.1% sales retention rate versus 2031 baseline sales in 2032 (the first full year of US LOE impact, a 26.9% YoY decline), followed by a constant 16.81% annual compounding decline in retention through 2035 (year four of US semaglutide LOE), bringing cumulative retention down to 42.1% of 2031 baseline sales by that year to reflect a straight-lined historical sales retention decrease. Even though SC Wegovy had flatlined to slightly decreasing TRx from June 2023 to January 2024, the release of Zepbound reaccelerated its growth rate to a rate comparable to what its initial launch growth exhibited as shown in Figure 5.4. While I don't apply that rationale to the same extent in the base case, I do forecast non-negative growth for the brand given SC Wegovy saw a continued elevated growth rate following Zepbound release for about a year and a half before returning to switching periods of flatlined growth and temporary boosts that elevate the level at which growth subsequently flatlined at once again.

Figure 5.4 – Drug TRx Progression:



(A graph depicting the TRx for key weight-loss and T2D GLP-1 drugs to be used as comparables in base, upside, and downside projections. It should be noted that while this data focuses solely on US TRx trends, I extrapolate this company-wide to the drug's total growth rates)

Given that Oral Wegovy is the principal drug in question and the core point of this investment memo, the following base case assumptions for the rest of the revenue build line items are outlined below:

- SC Wegovy: I assign a 10% growth rate for FY 2026 (based on the 10.3% June 2026 TRx percent change vs YA), followed by a 0.5% haircut to that growth each subsequent year from 2027 through 2031 as semaglutide LOEs expand, injectable and oral next-generation drugs are FDA-approved, and next-generation high-efficacy weekly injectables or high convenience orals/monthly injectables both cannibalize current generation drugs and attract incremental consumers to the market. For 2032 through 2035, I apply the brand originator sales retention framework and assign a 73.1% sales retention rate versus 2031 baseline sales in 2032 followed by a constant 16.81% annual compounding decline in sales through 2035.
- Saxenda: I assign a -50% growth rate to Saxenda for the rest of 2026 (based on the roughly -50% June 2026 TRx percent change vs YA) and a 33.4% sales rate decrease for

all following forecast periods. I derive this number from using the average between the sales retention framework's 16.81% annual sales decrease rate from years two to five post generic entrance and the current -50% sales given that liraglutide went off US patent in late 2024. The reason I don't apply the sales-retention framework to Saxenda is because liraglutide's US LOE occurred in November 2024, and the drug has already shown a 53% sales rate decrease from full-year 2024 sales to full-year 2025 sales. Therefore, given that the sales retention is almost double that of typical originator sales retention, I use the average between the typical sales rate decrease annually and what Saxenda has already exhibited. The reason for this assumption is due to Saxenda being a past-generation drug with extremely low relative efficacy and with the near-term release of next-generation drugs like CagriSema, Saxenda will become even more obsolete as cheaper generic semaglutide offerings overtake it.

- Ozempic: I assign a -9% growth rate for the rest of 2026 (based on the -8.7% June 2026 TRx percent change vs YA) and maintain that growth rate at -9% for each subsequent year up through 2031 as for the same reasoning as SC Wegovy: Patent LOE, generic competitors, and next-generation replacements. For 2032 through 2035, I apply the identical sales-retention framework: 73.1% retention versus 2031 baseline sales in 2032, declining at a constant 16.81% annual rate through 2035 to 42.1% cumulative retention.
- Victoza: I assign a -35% growth rate for the rest of 2026 (based on the -33% June 2026 TRx percent change vs YA) and apply the identical sales-retention framework with sales declining at a constant 24.9% annual rate through 2035, using the average of the sales-retention constant 16.81% annual sales decrease and Victoza's current 33% sales decrease due to liraglutide US LOE in late 2024.
- Rybelsus/Ozempic Pill: I assign a -17% growth rate for the rest of 2026 (based on the -16.7% June 2026 TRx percent change vs YA) but assign a strong 3% growth rate through 2031 due to the rebranding of Rybelsus to the Ozempic pill and the Halo Effect that the new, essentially identical drug will gain from having the world-renowned brand name attached to it. For simplicity, I treat Rybelsus and Ozempic Pill as the same drug, given that the former is being retired and essentially rebranded to the near-identical latter. While the jump from double-digit declines to mid-single-digit increases may seem stark, I argue it is justified given the reputation that Ozempic holds across the entire world and the indirect endorsements it has received from prominent people, including President Trump, Andy Cohen, and more.⁴⁸ For 2032 through 2035, because Rybelsus/Ozempic Pill contains the same active ingredient and faces the same US semaglutide LOE timeline, I

⁴⁸ Sara Ashley O'Brien, "What Housewife Isn't on Ozempic? How a Weight-Loss Craze Is Shaping Reality TV," *Wall Street Journal*, April 25, 2023, <https://www.wsj.com/style/beauty/what-housewife-isnt-on-ozempic-how-a-weight-loss-craze-is-shaping-reality-tv-afb21602>.

apply the identical sales-retention framework: 73.1% retention versus 2031 baseline sales in 2032, declining at a constant 16.81% annual rate through 2035 to 42.1% cumulative retention.

- Long-Acting Insulin Sales: I assign a 5% growth rate for the remainder of 2026 (based on the 5% mean sales QoQ growth rate of Q3 and Q4 over the past 3 years), and a lower -1% growth rate through 2035 as insulin becomes increasingly phased out by next-generation SC and oral T2D GLP-1s.
- Premix Insulin Sales: I assign a 1% growth rate for the remainder of 2026 (based on the 0.7% mean sales QoQ growth rate of Q3 and Q4 over the past 3 years), and a lower -1% growth rate through 2035 as insulin becomes increasingly phased out by next-generation SC and oral T2D GLP-1s.
- Fast-Acting Insulin Sales: I assign a 13% growth rate for the remainder of 2026 (based on the 13.1% mean sales QoQ growth rate of Q3 and Q4 over the past three years), and a lower -1% growth rate through 2035 as insulin becomes increasingly phased out by next-generation SC and oral T2D GLP-1s.
- Human Insulin Sales: I assign a 7% growth rate for the remainder of 2026 (based on the 6.9% mean sales QoQ growth rate of Q3 and Q4 over the past three years), and a lower -1% growth rate through 2035 as insulin becomes increasingly phased out by next-generation SC and oral T2D GLP-1s.
- Other Diabetes Care: I assign a -11% growth rate for the remainder of 2026 (based on the -11.2% mean sales QoQ growth rate of Q3 and Q4 over the past three years), and a flat 0% growth rate for the following forecasted years given the volatile increases and decreases in the line item overall and the line-item hovering near 500 million DKK for the past three years.
- Rare Blood Disorders: While the segment has previously shown weak performance, with 3.2% mean QoQ sales growth in Q3 and Q4 over the past three years, I assign a 5% growth rate for the remaining 2026 quarters and assign a 5% growth rate in 2027 before sloping it down to flat growth by 2035 as Denecimig becomes obsolete by future Hemophilia A innovations.
- Rare Endocrine Disorders: I assign a 40% growth rate for the remaining two quarters of 2026 (based on the 41.3% mean sales QoQ growth rate of Q3 and Q4 over the past three years), and a lower 8% growth rate for 2027 based on the strong YoY annual sales growth the line item has seen following FY 2023 lows, tapering it down to 0% growth by 2035.

- Other Rare Disease: I assign a 4% growth rate for the remaining two quarters of 2026 (based on the 3.9% mean sales QoQ growth rate of Q3 and Q4 over the past three years) and maintain that 4% growth in 2027 due to the recent return to sales growth the line item has experienced following FY 2024 lows, tapering it down to 0% growth by 2035.

Figure 5.5 – Base Case Revenue Build:

Revenue Build		All values are in DKK millions																			
Case																					
Base																					
Fiscal Period		FY 2020	FY 2021	FY 2022	FY 2023	FY 2024	FY 2025	Q1 2026	Q2 2026	Q3 2026 E	Q4 2026 E	FY 2026 E	FY 2027 E	FY 2028 E	FY 2029 E	FY 2030 E	FY 2031 E	FY 2032 E	FY 2033 E	FY 2034 E	FY 2035 E
Obesity and Diabetes Care Segment																					
Obesity Care Sales																					
Vegovy Sales		0	1,417	6,188	31,343	58,206	79,106	20,491	22,702	24,715	26,924	94,831	102,872	111,209	119,789	128,549	137,420	100,454	83,568	69,520	57,834
SC Vegovy Injectable		0	1,417	6,188	31,343	58,206	79,106	18,235	19,484	21,432	23,576	82,727	90,586	98,739	107,132	115,702	124,380	90,322	75,638	62,323	52,346
EUCAN		0	n/a	0	54	1,515	15,383	4,341	5,107	0	0	0	0	0	0	0	0	0	0	0	0
Region China		0	n/a	0	0	186	796	416	334	0	0	0	0	0	0	0	0	0	0	0	0
Emerging Markets		0	n/a	0	0	3,716	6,100	1,732	2,217	0	0	0	0	0	0	0	0	0	0	0	0
US Operations		0	n/a	6,194	29,430	45,770	51,015	9,457	9,836	0	0	0	0	0	0	0	0	0	0	0	0
APAC		0	n/a	0	0	1,011	5,812	1,653	1,530	0	0	0	0	0	0	0	0	0	0	0	0
Vegovy Pill		0	0	0	0	0	0	2,256	3,215	3,282	3,348	12,104	12,286	12,470	12,657	12,847	13,040	9,532	7,330	6,597	5,488
EUCAN		0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0
Region China		0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0
Emerging Markets		0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0
US Operations		0	0	0	0	0	0	2,256	3,941	0	0	0	0	0	0	0	0	0	0	0	0
APAC		0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0
Saxenda Sales		5,608	6,984	10,676	10,289	6,940	3,241	427	450	225	113	1,209	805	536	357	238	158	105	70	47	31
EUCAN		1,124	n/a	3,561	3,780	2,320	1,444	183	201	0	0	0	0	0	0	0	0	0	0	0	0
Region China		10	n/a	133	146	102	28	0	1	0	0	0	0	0	0	0	0	0	0	0	0
Emerging Markets		984	n/a	2,138	2,476	2,541	1,238	221	180	0	0	0	0	0	0	0	0	0	0	0	0
US Operations		3,230	n/a	4,368	3,306	777	268	3	47	0	0	0	0	0	0	0	0	0	0	0	0
APAC		280	n/a	476	581	600	283	14	21	0	0	0	0	0	0	0	0	0	0	0	0
Total Obesity Care Sales		5,608	8,400	16,864	41,632	65,146	82,347	20,912	23,152	24,940	27,036	96,040	103,677	111,745	120,146	128,787	137,578	100,559	83,638	69,567	57,865
GLP-1 Diabetes																					
Ozempic Sales		21,211	33,705	53,750	95,718	120,342	127,089	27,825	31,375	28,551	25,382	113,733	103,497	94,182	85,706	77,992	70,737	51,881	43,160	35,305	29,869
EUCAN		3,112	6,353	10,417	14,327	16,001	22,714	6,567	7,164	0	0	0	0	0	0	0	0	0	0	0	0
Region China		10	303	2,136	4,821	5,762	5,339	1,305	1,403	0	0	0	0	0	0	0	0	0	0	0	0
Emerging Markets		512	2,160	4,756	7,230	7,232	7,235	1,375	1,458	0	0	0	0	0	0	0	0	0	0	0	0
US Operations		16,650	23,168	36,750	63,010	84,201	88,467	17,730	20,536	0	0	0	0	0	0	0	0	0	0	0	0
APAC		327	1,681	3,631	6,330	7,086	3,214	648	798	0	0	0	0	0	0	0	0	0	0	0	0
Victoza Sales		18,747	15,054	12,322	8,664	5,482	3,020	334	161	118	76	703	533	400	300	226	163	127	96	72	54
EUCAN		4,251	3,527	2,724	2,195	1,422	894	71	54	0	0	0	0	0	0	0	0	0	0	0	0
Region China		1,033	1,544	1,478	1,256	975	601	127	93	0	0	0	0	0	0	0	0	0	0	0	0
Emerging Markets		1,811	1,655	1,470	1,428	1,283	1,050	104	24	0	0	0	0	0	0	0	0	0	0	0	0
US Operations		11,232	8,031	6,406	3,613	1,639	471	9	-8	0	0	0	0	0	0	0	0	0	0	0	0
APAC		360	297	244	201	97	204	23	12	0	0	0	0	0	0	0	0	0	0	0	0
Rybelsus Sales		1,873	4,838	11,239	16,750	23,301	22,093	4,572	5,228	4,339	3,602	17,741	18,273	18,821	13,366	19,967	20,566	15,034	12,507	10,404	8,895
EUCAN		36	289	1,174	4,232	7,116	7,065	1,538	1,454	0	0	0	0	0	0	0	0	0	0	0	0
Region China		0	0	63	131	511	620	26	-25	0	0	0	0	0	0	0	0	0	0	0	0
Emerging Markets		0	235	1,378	3,026	4,584	2,061	563	765	0	0	0	0	0	0	0	0	0	0	0	0
US Operations		1,826	4,243	8,011	11,060	10,735	8,833	1,594	2,272	0	0	0	0	0	0	0	0	0	0	0	0
APAC		11	71	133	301	275	3,514	635	761	0	0	0	0	0	0	0	0	0	0	0	0
Total GLP-1 Diabetes		41,831	53,597	83,371	123,132	149,125	152,202	32,737	36,794	33,008	29,660	132,163	122,303	115,403	105,332	98,165	91,709	67,043	55,762	46,381	38,573

(A snapshot of a portion of the revenue build, including all of the GLP-1 line-items and their projected top-line contributions for the base case.)

Figure 5.6 – Base Case Discounted Cash Flow Model:

Discounted Cash Flow Model

Novo Nordisk A/S (NVO:NYSE:NOVO B)

All values are in DKK millions except per-share data and ratios

Inputs		WACC + Terminal Rate Sensitivity Table					WACC + Exit Multiple Sensitivity Table						
Share Price (Friday, August 1, 2026)	303.85 kr.	2.0%	2.5%	3.0%	3.5%	4.0%	14.2x	14.7x	15.2x	15.7x	16.2x		
WACC	6.75%	5.79%	271.67 kr.	286.02 kr.	305.52 kr.	333.54 kr.	377.24 kr.	6.29%	300.53 kr.	303.02 kr.	305.52 kr.	308.01 kr.	310.50 kr.
Terminal Growth Rate	3%	6.29%	251.71 kr.	262.29 kr.	278.08 kr.	294.82 kr.	321.75 kr.	6.79%	271.21 kr.	273.69 kr.	276.08 kr.	278.48 kr.	280.95 kr.
Exit Multiple Method	15.2x	6.79%	235.09 kr.	243.13 kr.	253.29 kr.	266.95 kr.	284.56 kr.	7.29%	248.73 kr.	251.01 kr.	253.29 kr.	255.57 kr.	257.86 kr.
DKK/USD	0.15	7.29%	220.90 kr.	227.17 kr.	234.90 kr.	244.67 kr.	257.42 kr.	7.79%	230.53 kr.	232.71 kr.	234.90 kr.	237.08 kr.	239.27 kr.
Shares Outstanding (million)	4423.1	7.79%	208.66 kr.	213.54 kr.	219.57 kr.	227.00 kr.	236.40 kr.	7.79%	215.39 kr.	217.48 kr.	219.57 kr.	221.66 kr.	223.75 kr.

Actual																Projected									
	FY '21	FY '22	FY '23	FY '24	FY '25	FY '26	FY '27	FY '28	FY '29	FY '30	FY '31	FY '32	FY '33	FY '34	FY '35										
EBIT	58,844	74,809	102,574	128,339	127,658	122,976	129,838	129,771	130,180	130,377	132,085	105,791	93,713	83,578	75,039										
Tax Rate	19.2%	19.8%	20.1%	20.8%	21.5%	18.9%	21.0%	21.0%	21.0%	21.0%	21.0%	21.0%	21.0%	21.0%	21.0%										
NOPAT	47,405	60,146	82,004	101,899	100,173	99,037	102,572	102,519	102,842	103,472	104,347	83,575	74,034	66,026	59,281										
Add: Depreciation and	5,311	6,553	7,289	8,545	14,597	14,255	13,822	13,815	13,858	13,943	14,061	11,262	9,976	8,897	7,989										
Less: Capex	-6,335	-12,146	-25,806	-47,164	-60,140	-59,469	-34,360	-23,784	-27,134	-25,766	-23,819	-19,077	-16,899	-15,072	-13,532										
Less: Change in Net Working	7,360	17,132	18,907	15,260	-10,868	-20,405	7,694	-23	140	274	380	-3,017	-4,142	-3,476	-2,328										
Unlevered Cash Flow	54,341	71,685	82,394	78,540	43,762	33,417	89,727	86,527	89,706	91,922	94,969	66,743	62,969	56,376	50,805										

Discount Period																
Discount Rate						0.42	1.42	2.42	3.42	4.42	5.42	6.42	7.42	8.42	9.42	
Present Value of UFCF						1.03	1.10	1.17	1.25	1.34	1.43	1.52	1.63	1.74	1.86	
Total Present Value of UFCF						32,515	81,756	73,830	71,677	68,780	66,543	43,793	38,690	32,438	27,377	537,398

Perpetuity Growth Model															
Terminal Free Cash Flow	50,809														
Perpetuity Growth Rate	3.00%														
Terminal Value	1,381,801														
PV of Terminal Value	744,533														
PV of FCF	537,398														
Enterprise Value	1,281,931														
Implied Exit Multiple	19.4x														
Exit Multiple															
Terminal Year EBIT	75,039														
Exit Multiple (TEV/EBIT)	15.2x														
Terminal Value	1,140,598														
PV of Terminal Value	614,569														
PV of FCF	537,398														
Enterprise Value	1,151,968														
Implied Perpetuity Growth	2.2%														

Exit Multiple			Perpetuity Growth Model		
Enterprise	1,151,968		Enterprise	1,281,931	
Total Debt	140,127		Total Debt	140,127	
Min. Interest	-		Min. Interest	-	
Cash	44,982		Cash	44,982	
Equity Value	1,056,823		Equity Value	1,056,786	
Implied	238.61 kr.		Implied	267.95 kr.	
Implied	\$37.99		Implied	\$40.19	
Blended Share Price			Blended Share Price		
Method	Implied Share Price	% Weight	Method	Implied Share Price	% Weight
Exit Multiple	238.61	50%	Exit Multiple	238.61	50%
Growth	267.95	50%	Growth	267.95	50%
Blended Share Price			Blended Share Price		
Current Share			Current Share		
Upside/(Downside)			Upside/(Downside)		

(A snapshot of my DCF for the base case, which yields an implied share price of \$37.99, or a 19.6% downside to NVO's share price as of August 7th, 2026.)

Upside Case

For the upside case, I assume Oral Wegovy's prescription TRx rate to reaccelerate even as Foundayo scales with increased marketing, effectively making my thesis null. As such, I assign a 15% top-line TRx growth rate for the rest of 2026 based on the reacceleration that Wegovy experienced with ~15% monthly TRx growth following the release of Zepbound. For the projected years through 2031, I flatline a lower 8.25% growth rate annually in top-line sales for Oral Wegovy, which sits between the base case annual growth rate and upside case quarterly growth rates, representing a strong but moderating growth cycle for the drug. Following semaglutide US patent LOE in 2031, however, I decrease that growth rate by applying the same sales-retention framework as previously employed: 73.1% retention versus 2031 baseline sales in

2032, declining at a constant 16.81% annual rate through 2035 to 42.1% cumulative retention to represent cannibalization effects from generics with similar efficacy and lower prices.

Given that Oral Wegovy is the principal drug in question and the core point of this investment memo, the following upside case assumptions for the rest of the revenue build line items are outlined below:

- SC Wegovy: I assign the base case 10% growth rate for FY 2026 and maintain that 10% growth rate each subsequent year through 2031 to reflect SC Wegovy maturing but still holding on to growth. From 2032 to 2035, I decrease that growth rate by applying the same sales-retention framework as previously employed: 73.1% retention versus 2031 baseline sales in 2032, declining at a constant 16.81% annual rate through 2035 to 42.1% cumulative retention to represent cannibalization effects from generics with similar efficacy and lower prices.
- Saxenda: I am maintaining the base case -50% growth rate to Saxenda for the rest of 2026 (based on the roughly -50% June 2026 TRx percent change vs YA) and a 33.4% sales rate decrease for all following forecasted periods. The reason for this assumption is due to Saxenda being a past-generation drug with extremely low relative efficacy and with the near-term release of next-generation drugs like CagriSema, Saxenda will become even more obsolete as cheaper generic semaglutide offerings overtake it.
- Ozempic: I assign the same initial base case -9% growth rate for the rest of 2026 and include a 0.5% positive haircut for each subsequent year to represent still decreasing sales contribution from Ozempic but at lesser scale through 2035 due to brand-name recognition aspect that halts further acceleration in sales declines. For 2032 through 2035, I apply the same base sales-retention framework: 73.1% retention versus 2031 baseline sales in 2032, declining at a constant 16.81% annual rate through 2035 to 42.1% cumulative retention to represent cannibalization effects from generics with similar efficacy and lower prices.
- Victoza: I assign the same base case -35% growth rate for the rest of 2026 and apply the identical sales-retention framework with sales declining at a constant 24.9% annual rate through 2035, using the average of the sale-retention constant 16.81% annual sales decrease and Victoza's current 33% sales decrease.

- Rybelsus/Ozempic Pill: I assign a -12% growth rate for the rest of 2026 (base case -17% growth + 5% Ozempic brand-name initial scaling effect) but assign a stronger 10% growth rate for the forecast years through 2031 due to the rebranding of Rybelsus to the Ozempic pill and the Halo Effect that the new, essentially identical drug will gain. For simplicity, I treat Rybelsus and Ozempic Pill as the same drug outside of the name change. For 2032 through 2035, I apply the same, slightly more favorable retention framework as Ozempic: 73.1% retention versus 2031 baseline sales in 2032, declining at a constant 16.81% annual rate through 2035 to 42.1% cumulative retention to represent cannibalization effects from generics with similar efficacy and lower prices.
- Long-Acting Insulin Sales: I assign a 3% (5% base case + -2% oral cannibalization effect) growth rate for the remainder of 2026, and a lower 1% (-1% base case + 2% upside spread) growth rate for the following forecasted years as insulin becomes increasingly phased out by next-generation SC and oral T2D GLP-1s.
- Premix Insulin Sales: I assign a -1% growth rate for the remainder of 2026 (1% base case – 2% oral cannibalization effect), and project Premix to be flat at 1% (-1% base case + 2% upside spread) growth rate for the following forecasted years as insulin becomes increasingly phased out by next-generation SC and oral T2D GLP-1s.
- Fast-Acting Insulin Sales: I assign an 11% (13% base case + -2% oral cannibalization effect) growth rate for the remainder of 2026, and a lower 1% (-1% base case + 2% upside spread) growth rate for the following forecasted years given the weaker growth compared to Premix and same exposure to T2D GLP-1 replacement.
- Human Insulin Sales: I assign a 5% (7% base case + -2% oral cannibalization effect) growth rate for the remainder of 2026, and a lower 1% (-1% base case + 2% upside spread) growth rate for the following forecasted years given the weaker growth compared to Fast-Acting but same exposure to T2D GLP-1 replacement.
- Other Diabetes Care: I assign the same base case -11% growth rate for the remainder of 2026 and flat 0% growth rate for the following forecasted years given the volatile increases and decreases in the line item overall.
- Rare Blood Disorders: I assign the same base case 5% growth rate for the remaining 2026 quarters and full year 2027 before sloping it down to flat growth by 2035.
- Rare Endocrine Disorders: I assign the same base case 40% growth rate for the remaining two quarters of 2026 and a lower 8% growth rate for 2027 that moderates to 0% growth by 2035 as it exhibits a cyclical boom and bust cycle.

- Other Rare Disease: I assign the same base case 4% growth rate for the remaining two quarters of 2026 and maintain that 4% growth rate for 2027 before moderating it down to 0% by 2035.

Figure 5.7 – Upside Case Revenue Build:

Revenue Build		All values are in DKK millions																																							
Case																																									
Upside																																									
Period		FY 2020	FY 2021	FY 2022	FY 2023	FY 2024	FY 2025	Q1 2026	Q2 2026	Q3 2026	Q4 2026	FY 2026 E	FY 2027 E	FY 2028 E	FY 2029 E	FY 2030 E	FY 2031 E	FY 2032 E	FY 2033 E	FY 2034 E	FY 2035 E																				
Obesity and Diabetes Care Segment																																									
Obesity Care Sales																																									
Wegovy Sales		0	1,417	6,188	31,343	58,206	79,106	20,491	22,702	25,133	27,831	96,158	105,538	115,838	127,146	139,562	153,196	119,986	93,961	77,501	64,473																				
SE Wegovy Injectable		0	1,417	6,188	31,343	58,206	79,106	18,235	19,404	21,432	23,576	82,727	91,000	100,100	100,100	121,211	133,233	97,393	81,021	67,402	56,071																				
EUCAN		0	n/a	54	1,913	7,513	15,363	4,341	5,707	0	0	0	0	0	0	0	0	0	0	0	0																				
Region China		0	n/a	0	0	0	796	796	416	0	0	0	0	0	0	0	0	0	0	0	0																				
Emerging Markets		0	n/a	0	0	0	3,716	6,100	1,732	2,217	0	0	0	0	0	0	0	0	0	0	0																				
US Operations		0	n/a	6,134	23,430	45,770	51,015	9,433	9,636	0	0	0	0	0	0	0	0	0	0	0	0																				
APAC		0	n/a	0	0	0	1,011	5,812	1,553	0	0	0	0	0	0	0	0	0	0	0	0																				
Wegovy Pill		0	0	0	0	0	0	2,258	3,219	3,701	4,256	13,431	14,539	15,738	17,036	18,442	19,963	14,593	12,140	10,099	8,402																				
EUCAN		0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0																				
Region China		0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0																				
Emerging Markets		0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0																				
US Operations		0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0																				
APAC		0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0	0																				
Savenda Sales		5,608	6,984	10,676	10,289	6,940	3,241	421	451	225	113	1,209	805	536	357	238	158	105	70	47	31																				
EUCAN		1,124	n/a	3,561	3,780	2,920	1,444	163	201	0	0	0	0	0	0	0	0	0	0	0	0																				
Region China		10	n/a	133	146	102	28	0	0	0	0	0	0	0	0	0	0	0	0	0	0																				
Emerging Markets		984	n/a	2,138	2,476	2,541	1,238	221	180	0	0	0	0	0	0	0	0	0	0	0	0																				
US Operations		3,230	n/a	4,368	3,306	777	268	3	47	0	0	0	0	0	0	0	0	0	0	0	0																				
APAC		260	n/a	476	581	800	253	14	21	0	0	0	0	0	0	0	0	0	0	0	0																				
Total Obesity Care Sales		5,608	6,400	16,984	41,632	65,146	82,347	20,312	23,352	25,358	27,944	97,366	106,343	116,374	127,503	139,800	153,354	112,092	93,232	77,548	64,500																				
GLP-1 Diabetes																																									
Ozempic Sales		21,211	33,705	59,750	95,718	120,342	127,089	27,825	31,375	28,551	25,982	113,733	104,066	95,740	88,560	82,361	77,007	56,232	46,830	38,957	32,409																				
EUCAN		3,112	6,393	10,417	14,327	16,001	22,774	6,567	7,184	0	0	0	0	0	0	0	0	0	0	0	0																				
Region China		10	303	2,196	4,821	5,762	5,399	1,305	1,409	0	0	0	0	0	0	0	0	0	0	0	0																				
Emerging Markets		512	2,860	4,756	7,230	7,232	7,235	1,375	1,458	0	0	0	0	0	0	0	0	0	0	0	0																				
US Operations		16,650	23,168	38,750	63,010	84,201	88,467	17,730	20,538	0	0	0	0	0	0	0	0	0	0	0	0																				
APAC		927	1,691	3,631	6,330	7,088	3,214	840	766	0	0	0	0	0	0	0	0	0	0	0	0																				
Victoza Sales		18,147	15,054	12,322	8,884	5,482	3,020	334	18	118	76	709	533	400	300	226	169	127	96	72	54																				
EUCAN		4,251	3,527	2,724	2,166	1,422	694	71	54	0	0	0	0	0	0	0	0	0	0	0	0																				
Region China		1,033	1,544	1,478	1,256	975	801	127	98	0	0	0	0	0	0	0	0	0	0	0	0																				
Emerging Markets		1,811	1,655	1,470	1,428	1,289	1,050	104	24	0	0	0	0	0	0	0	0	0	0	0	0																				
US Operations		11,232	8,031	6,406	3,613	1,693	471	9	-8	0	0	0	0	0	0	0	0	0	0	0	0																				
APAC		360	297	244	201	97	204	12	12	0	0	0	0	0	0	0	0	0	0	0	0																				
Rybelsus Sales		1,873	4,838	11,299	18,750	23,301	22,093	4,572	5,228	4,601	4,049	18,449	20,294	22,324	24,556	27,011	29,713	21,720	18,069	15,031	12,505																				
EUCAN		36	289	1,174	4,232	7,136	7,065	1,538	1,454	0	0	0	0	0	0	0	0	0	0	0	0																				
Region China		0	0	63	131	511	620	26	-25	0	0	0	0	0	0	0	0	0	0	0	0																				
Emerging Markets		0	235	1,378	3,026	4,584	2,061	569	765	0	0	0	0	0	0	0	0	0	0	0	0																				
US Operations		1,826	4,243	8,071	11,060	10,795	8,633	1,504	2,273	0	0	0	0	0	0	0	0	0	0	0	0																				
APAC		11	71	771	3,011	275	314	935	78	0	0	0	0	0	0	0	0	0	0	0	0																				
Total GLP-1 Diabetes		41,831	53,897	83,371	123,137	145,125	152,002	32,793	36,794	33,720	30,107	132,891	124,852	116,456	113,415	105,558	106,889	78,193	64,394	54,061	44,361																				

(A snapshot of my DCF for the upside case, which yields an implied share price of \$42.32, or a 10.4% downside to NVO's share price as of August 7th, 2026.)

Downside Case

For the downside case, I assume Oral Wegovy's prescription TRx rate to begin to decelerate, on a similar trajectory to SC Wegovy following its post-launch TRx surge as Foundayo cannibalizes significant market share from Oral Wegovy due to increased marketing backing by Eli Lilly now that PBM and prescriber Foundayo relationships are established, effectively fulfilling my thesis of Foundayo overshadowing. As such, I assign a -9% top-line TRx growth rate for the rest of 2026 based on the -9.3% average monthly TRx growth rate that SC Wegovy experienced following the easing of its supply constraints in late 2022. Since SC Wegovy declines happened without any major competitor in the market, given that Zepbound began scaling in late 2023, I considered lowering that negative growth rate to -15% for the remainder of 2026 to represent the key competitor that Foundayo is to Oral Wegovy. However, I maintained it at slightly higher than historical injectable precedents due to the dual growth scenario and reacceleration in growth that SC Wegovy experienced following Zepbound's initial scaling. For the rest of the projection years through 2031, I flatline a 1% growth rate annually in top-line sales for Oral Wegovy given the comparable average monthly growth of 2.3% that SC Wegovy experienced from August 2024 to June 2026 after becoming accustomed to Zepbound's presence in the market and 1.5% base case 2027-2031 Oral Wegovy growth rate. Following semaglutide US patent LOE in 2031, however, I decrease that growth rate by applying the same sales-retention framework as previously employed: 73.1% retention versus 2031 baseline sales in

2032, declining at a constant 16.81% annual rate through 2035 to 42.1% cumulative retention to represent cannibalization effects from generics with similar efficacy and lower prices.

Given that Oral Wegovy is the principal drug in question and the core point of this investment memo, the following downside case assumptions for the rest of the revenue build line items are outlined below:

- SC Wegovy: I assign the same base/upside case 10% growth rate for FY 2026, followed by a haircut of 1% (0.5% base case, 0% upside case) to annual growth starting 2027 and continuing for each subsequent year through 2031 as semaglutide LOEs expand, injectable and oral next-generation drugs are FDA-approved, and consumers switch away from current-generation drugs towards next-generation high-efficacy weekly injectables or high convenience orals/monthly injectables. From 2032 to 2035, I apply the same base and upside case sales-retention framework: 73.1% retention versus 2031 baseline sales in 2032, declining at a constant 16.81% annual rate through 2035 to 42.1% cumulative retention to represent cannibalization effects from generics with similar efficacy and lower prices.
- Saxenda: I assign the same base/upside case -50% growth rate to Saxenda for the rest of 2026 and a 33.4% sales rate decrease for all following forecasted years.
- Ozempic: I assign the same base/upside case -9% growth rate for the rest of 2026 followed by a -0.5% (0% base case, +0.5% upside case) haircut each subsequent year through 2031 for the same reasoning as SC Wegovy: Patent LOE, generic competitors, and next-generation replacements. For 2032 through 2035, I apply the same base and upside case sales-retention framework: 73.1% retention versus 2031 baseline sales in 2032, declining at a constant 16.81% annual rate through 2035 to 42.1% cumulative retention to represent cannibalization effects from generics with similar efficacy and lower prices.
- Victoza: I assign the base/upside case -35% growth rate for the rest of 2026 and apply the identical sales-retention framework with sales declining at a constant -24.9% annual rate through 2035, using the average of the sale-retention constant 16.81% annual sales decrease and Victoza's current 33% sales decrease.
- Rybelsus/Ozempic Pill: I assign a -14% growth rate for the rest of 2026 (base case -17% growth + smaller 3% Ozempic brand-name initial scaling effect) and assign a flat 0%

(base case 5% + -5% Foundayo oral T2D indication cannibalization effect) growth rate for the forecast years through 2031. For 2032 through 2035, I apply the same base and upside case sales-retention framework: 73.1% retention versus 2031 baseline sales in 2032, declining at a constant 16.81% annual rate through 2035 to 42.1% cumulative retention to represent cannibalization effects from generics with similar efficacy and lower prices.

- Long-Acting Insulin Sales: I assign the same upside case 3% (5% base case + -2% oral cannibalization effect) growth rate for the remainder of 2026, and a lower -3% (-1% base case + -2% downside spread) growth rate for the following forecasted years as insulin becomes increasingly phased out by next-generation SC and oral T2D GLP-1s.
- Premix Insulin Sales: I assign the same upside case -1% growth rate for the remainder of 2026 (1% base case + -2% oral cannibalization effect), and project Premix to be flat at -3% (-1% base case + -2% downside spread) growth rate for the following forecasted years as insulin becomes increasingly phased out by next-generation SC and oral T2D GLP-1s.
- Fast-Acting Insulin Sales: I assign the same upside case 11% (13% base case + -2% oral cannibalization effect) growth rate for the remainder of 2026, and a lower -3% (-1% base case + -2% downside spread) growth rate for the following forecasted years.
- Human Insulin Sales: I assign the same upside case 5% (7% base case + -2% oral cannibalization effect) growth rate for the remainder of 2026, and a lower -3% (-1% base case + -2% downside spread) growth rate for the following forecasted years given the weaker growth compared to Fast-Acting but same exposure to T2D GLP-1 replacement.
- Other Diabetes Care: I assign the same base/upside case -11% growth rate for the remainder of 2026 and flat 0% growth rate for the following forecasted years given the volatile increases and decreases in the line item overall.
- Rare Blood Disorders: I assign the same base/upside case 5% growth rate for the remaining 2026 quarters as Denecimig is released and maintain that 5% growth rate in 2027 before sloping it down to flat growth by 2035 as Denecimig becomes obsolete by future Hemophilia A innovations.
- Rare Endocrine Disorders: I assign the same base/upside case 40% growth rate for the remaining two quarters of 2026 and a lower 8% growth rate in 2027 that tapers to 0% growth by 2035.

- Other Rare Disease: I assign the same base/upside case 4% growth rate for the remaining two quarters of 2026 and maintain that 4% growth in 2027 due to the recent return to sales growth the line item has experienced following FY 2024 lows, moderating it down to 0% growth by 2035.

Figure 5.9 – Downside Case Revenue Build:

Revenue Build		All values are in DKK millions																						
Case																								
Downside																								
Fiscal Period		FY 2020	FY 2021	FY 2022	FY 2023	FY 2024	FY 2025	Q1 2026	Q2 2026	Q3 2026 E	Q4 2026 E	FY 2026 E	FY 2027 E	FY 2028 E	FY 2029 E	FY 2030 E	FY 2031 E	FY 2032 E	FY 2033 E	FY 2034 E	FY 2035 E			
Obesity and Diabetes Care Segment																								
Obesity Care Sales																								
Vegovy Sales		0	1,417	6,188	31,343	58,206	73,106	20,491	22,702				24,361	26,240	33,794	111,350	108,676	115,606	121,972	127,610	93,283	77,602	64,557	53,705
SC Vegovy Injectable		0	1,417	6,188	31,343	58,206	73,106	20,451	22,702				21,432	23,576	32,727	90,172	37,386	104,203	110,456	115,378	84,780	70,523	58,673	48,870
EUCAN		0	n/a	54	1,913	7,513	15,383	4,341	5,707															
Region China		0	n/a	0	0	196	796	416	334															
Emerging Markets		0	n/a	0	0	3,716	6,100	1,732	2,217															
US Operations		0	n/a	6,134	29,430	45,770	51,015	3,437	3,636															
APAC		0	n/a	0	0	1,011	5,812	1,653	1,550															
Vegovy Pill		0	0	0	0	0	0	2,256	3,219				2,328	2,665	11,067	11,178	11,230	11,403	11,517	11,632	8,503	7,073	5,884	4,895
EUCAN		0	0	0	0	0	0	0	0															
Region China		0	0	0	0	0	0	0	0															
Emerging Markets		0	0	0	0	0	0	0	0															
US Operations		0	0	0	0	0	0	2,256	3,141															
APAC		0	0	0	0	0	0	0	0															
Savendra Sales		5,608	6,384	10,676	10,289	6,340	3,241	427	450				225	113	1,209	805	536	357	238	158	105	70	47	31
EUCAN		1,124	n/a	3,561	3,780	2,320	1,444	163	201															
Region China		10	n/a	133	146	162	28	0	1															
Emerging Markets		984	n/a	2,138	2,476	2,541	1,238	221	180															
US Operations		3,230	n/a	4,368	3,306	777	268	3	47															
APAC		260	n/a	476	591	600	383	14	21															
Total Obesity Care Sales		5,608	8,400	16,864	41,632	65,146	82,347	20,912	23,152	24,586	26,353	95,003	102,155	109,212	115,963	122,210	127,768	93,388	77,672	64,604	53,736			
GLP-1 Diabetes																								
Ozempic Sales		21,211	33,705	59,750	95,718	120,342	127,089	27,825	31,375	28,551	25,382	113,733	102,928	92,635	82,309	73,789	65,303	47,737	39,712	33,036	27,483			
EUCAN		3,112	6,393	10,417	14,327	16,001	22,774	6,567	7,184															
Region China		10	303	2,196	4,621	5,762	5,339	1,305	1,409															
Emerging Markets		512	2,160	4,756	7,230	7,232	7,232	1,375	1,458															
US Operations		16,650	23,168	38,750	63,070	84,201	88,467	17,730	20,538															
APAC		327	1,681	3,631	6,330	7,086	3,214	849	786															
Victoza Sales		18,747	15,054	12,322	8,664	5,482	3,020	334	18	118	76	709	533	400	300	226	169	127	96	72	54			
EUCAN		4,251	3,527	2,724	2,166	1,422	634	71	54															
Region China		1,033	1,544	1,478	1,256	975	601	127	95															
Emerging Markets		1,811	1,655	1,470	1,428	1,289	1,050	104	24															
US Operations		11,232	8,031	6,406	3,613	1,899	471	9	-8															
APAC		360	297	244	201	97	204	23	12															
Rybelsus Sales		1,873	4,838	11,239	18,750	23,301	22,093	4,572	5,228	4,436	3,867	18,163	18,163	18,163	18,163	18,163	18,163	13,277	11,045	9,188	7,644			
EUCAN		36	289	1,714	4,232	7,136	7,085	1,536	1,454															
Region China		0	0	63	131	511	620	26	-25															
Emerging Markets		0	235	1,378	3,028	4,584	2,061	589	765															
US Operations		1,826	4,243	8,011	11,060	10,795	8,633	1,584	2,273															
APAC		11	71	133	301	275	334	835	161															
Total GLP-1 Diabetes		41,831	53,597	83,371	123,132	143,125	152,202	32,731	36,784	33,165	29,325	132,605	121,624	111,198	101,372	92,177	83,635	61,141	50,853	42,297	35,181			

(A snapshot of a portion of the revenue build, including all of the GLP-1 line-items and their projected top-line contributions for the downside case.)

Figure 5.10 – Downside Case Discounted Cash Flow Model:

Discounted Cash Flow Model														
Novo Nordisk A/S (XCSE:NOVO B)														
All values are in DKK millions except per-share data and ratios														
Inputs			WACC + Terminal Rate Sensitivity Table								WACC + Exit Multiple Sensitivity Table			
Share Price (Friday, August 7, 2026)	303.85 kr.		2.0%		2.5%		3.0%		3.5%		14.2x		14.7x	15.2x
WACC	6.73%		5.79%	248.79 kr.	251.77 kr.	279.39 kr.	304.73 kr.	344.23 kr.	5.79%	274.87 kr.	277.93 kr.	279.39 kr.	281.85 kr.	283.32 kr.
Terminal Growth Rate	3%		6.29%	230.88 kr.	240.24 kr.	252.71 kr.	269.65 kr.	293.99 kr.	6.29%	248.38 kr.	250.54 kr.	252.71 kr.	254.87 kr.	257.03 kr.
Exit Multiple Method	15.2x		6.79%	215.88 kr.	222.85 kr.	232.04 kr.	244.02 kr.	260.31 kr.	6.79%	227.90 kr.	229.97 kr.	232.04 kr.	234.11 kr.	236.18 kr.
DKK/USD	0.15		7.29%	202.88 kr.	208.35 kr.	215.34 kr.	224.17 kr.	235.70 kr.	7.29%	211.38 kr.	213.36 kr.	215.34 kr.	217.32 kr.	219.30 kr.
Shares Outstanding (million)	442.91		7.79%	191.46 kr.	195.97 kr.	201.42 kr.	208.14 kr.	216.63 kr.	7.79%	197.62 kr.	199.62 kr.	201.42 kr.	203.31 kr.	205.21 kr.
Actual														
	FY '21	FY '22	FY '23	FY '24	FY '25	FY '26	FY '27	FY '28	FY '29	FY '30	FY '31	FY '32	FY '33	FY '34
EBIT	58,644	74,809	102,574	128,339	127,658	122,414	128,107	126,530	125,049	123,582	122,051	97,449	85,964	76,277
Tax Rate	19.2%	19.6%	20.1%	20.6%	21.5%	19.5%	21.0%	21.0%	21.0%	21.0%	21.0%	21.0%	21.0%	21.0%
NOPAT	47,405	60,146	82,004	101,899	100,173	98,586	101,205	99,959	98,788	97,630	96,420	76,385	67,311	60,259
Add: Depreciation and	5,311	6,553	7,289	8,545	14,597	14,190	13,638	13,470	13,312	13,156	12,993	10,374	9,151	8,120
Less: Capex	-6,335	-12,146	-25,806	-47,164	-60,140	-53,147	-33,902	-29,040	-26,064	-24,311	-22,009	-17,573	-15,502	-13,755
Less: Change in Net Working	7,360	17,132	18,907	15,260	-10,868	-20,857	7,652	-541	-508	-503	-525	-8,437	-3,939	-3,322
Unlevered Cash Flow	54,341	71,685	82,394	78,540	43,762	32,772	88,493	83,847	85,528	85,971	86,878	61,349	57,622	51,302
														45,933
Projected														
	FY '21	FY '22	FY '23	FY '24	FY '25	FY '26	FY '27	FY '28	FY '29	FY '30	FY '31	FY '32	FY '33	FY '34
EBIT	58,644	74,809	102,574	128,339	127,658	122,414	128,107	126,530	125,049	123,582	122,051	97,449	85,964	76,277
Tax Rate	19.2%	19.6%	20.1%	20.6%	21.5%	19.5%	21.0%	21.0%	21.0%	21.0%	21.0%	21.0%	21.0%	21.0%
NOPAT	47,405	60,146	82,004	101,899	100,173	98,586	101,205	99,959	98,788	97,630	96,420	76,385	67,311	60,259
Add: Depreciation and	5,311	6,553	7,289	8,545	14,597	14,190	13,638	13,470	13,312	13,156	12,993	10,374	9,151	8,120
Less: Capex	-6,335	-12,146	-25,806	-47,164	-60,140	-53,147	-33,902	-29,040	-26,064	-24,311	-22,009	-17,573	-15,502	-13,755
Less: Change in Net Working	7,360	17,132	18,907	15,260	-10,868	-20,857	7,652	-541	-508	-503	-525	-8,437	-3,939	-3,322
Unlevered Cash Flow	54,341	71,685	82,394	78,540	43,762	32,772	88,493	83,847	85,528	85,971	86,878	61,349	57,622	51,302
														45,933
Discount Period														
Discount Rate						0.42	1.42	2.42	3.42	4.42	5.42	6.42	7.42	8.42
Present Value of UFCF						1.03	1.10	1.17	1.25	1.34	1.43	1.52	1.63	1.74
Total Present Value of UFCF						31,888	80,631	71,543	68,339	64,327	60,873	40,254	35,405	29,518
														24,749
														507,527
Perpetuity Growth Model														
Terminal Free Cash Flow	45,933													
Perpetuity Growth Rate	3.00%													
Terminal Value	1,243,180													
PV of Terminal Value	673,075													
PV of FCF	607,527													
Enterprise Value	1,180,602													
Implied Exit Multiple	18.4x													
Exit Multiple														
Terminal Year EBIT	68,071													
Exit Multiple (TEV/EBIT)	15.2x													
Terminal Value	1,034,675													
PV of Terminal Value	557,497													
PV of FCF	507,527													
Enterprise Value	1,065,024													
Implied Perpetuity Growth	2.2%													
Exit Multiple														
Enterprise	1,065,024													
Total Debt	140,127													
Min. Interest	-													
Cash	44,982													
Equity Value	969,875													
Implied	218.98 kr.													
Implied	\$32.95													
Perpetuity Growth Model														
Enterprise	1,065,024													
Total Debt	140,127													
Min. Interest	-													
Cash	44,982													
Equity Value	969,875													
Implied	218.98 kr.													
Implied	\$32.95													
Exit Multiple														
Method	Implied Share Price													
Exit Multiple	218.98													
Growth	245.07													
Blended Share Price	232.03 kr.													
Current Share	303.85													
Upside/Downside	-23.6%													

(A snapshot of my DCF for the downside case, which yields an implied share price of \$34.80, or a 26.3% downside to NVO's share price as of August 7th, 2026.)

In summary, my three-case analysis of Novo Nordisk produced implied share prices of \$37.99, \$42.32, and \$34.80, leading to implied downsides of -19.6%, -10.4%, and -26.3% for the base, upside, and downside cases, respectively. To determine each share price, I utilized all of the same DCF inputs including a 3% terminal rate (2% above consensus to compensate for pipeline non-inclusion), 6.79% WACC (consensus was 7.5%), an exchange rate of \$0.15 USD to 1 Danish Kroner, and the share price and total shares outstanding as of August 7, 2026. For terminal value, I used both the exit multiple method, using a mean 15.2x TEV/EBIT multiple from a comp set of every public company with drugs listed in the appendix, and a Gordon growth method, utilizing a 3% terminal rate off forecast year 10 EBIT, applying equal weight to

the two methods. Notably, all of my cases project double-digit percent downside, and my downside case implies a share price below the 5-year low of \$35.12.

While this may seem very pessimistic, I argue that is validated and believe the rationale for the current 5-year low price serves as precedent for the stock reaching a new low. The 5-year low, as noted previously, was reached in late March of 2026 because of CagriSema's failure to reach its primary endpoint of non-inferiority to tirzepatide in the REDEFINE-4 trial. Furthermore, NVO's stock suffered its historic 70% plummet due to superior competitor drugs, Eli Lilly's at that, cannibalizing its domineering market share in the US and abroad. Given that the downside case implies the realization of a similar TRx and oral market share cannibalization by Foundayo over Novo Nordisk's current dominant position, I argue the three cases and the downside case's new 5-year low are justified.

Key Open Questions

Oral Pipeline Competitors

Three key questions will determine whether Novo Nordisk will ultimately cede market share to Foundayo as it did with Ozempic to Mounjaro and Wegovy to Zepbound, or whether it will be able to hold on to its domineering Oral market share. As Retatrutide marks a new era for efficacy and oral GLP-1s offering a new frontier for consumer convenience, the GLP-1 market is entering a structurally different era where a change in the most important input to top-line sales, consumer demand, is likely to change and adapt to the new modes of weight loss. Importantly, while there remain several pipeline drugs which could have a significant effect on which

company holds the majority oral GLP-1 market share, including Structure Therapeutics' [Aleniglipron](#), AstraZeneca's [Elecoglipron](#), or Viking Therapeutics' [VK2735](#), none of these drugs are likely to gain FDA approval by the end of 2026. Therefore, the key question regarding future market share of the oral GLP-1 market specifically strongly relies on which restriction consumers view as the least intrusive out of the two weight loss drugs.

Compounding Companies' Future Prevalence

Although not discussed heavily in this paper, another important question for NVO's future price action revolves around the development of US compounders and whether they pose a significant threat to Novo Nordisk's oral TRx development. On Monday, January 12, at the annual J.P. Morgan health conference in San Francisco, Novo Nordisk's current CEO Mike Doustdar highlighted that as many as 1.5 million Americans are currently using compounded GLP-1 drugs. Given that inflation remains consistently above Fed's 2% target and the average consumer is increasingly feeling their wallet pressured by rising gas prices among many other goods, the common GLP-1 user may instead begin to favor compounders' oral formulations, even if they don't match efficacy or tolerability. For the purposes of this report's short recommendation, compounder oral market cannibalization would be a strong tailwind for the continued decrease in Oral Wegovy's TRx growth rate. In a worst-case scenario, if wallet pressure increased even further due to continued restriction on Hormuz and Bab el-Mandeb Strait Oil flows, AI Hyperscaler data center build-out, or a combination of factors, compounded drugs could become so appealing to consumers that TRx for Oral Wegovy could reach an inflection point and begin decreasing in the US.

While this remains speculation, the impact of continuously elevated inflation in the US is a key consideration for whether consumers will continue to expend their diminishing real disposable income on brand name GLP-1s if cheaper alternatives exist. A mitigant to this compounder cannibalization scenario does exist however in congressional legislation, specifically the SAFE Drugs Act of 2026, that could significantly limit the amount of illegal compounding. The bill, which advanced to the Senate floor following its passing in the HELP committee three weeks ago, could severely limit the prominence that compounders hold currently and in the near future within the GLP-1 market by requiring compounders to submit reports detailing serious adverse events associated with their drugs to the FDA, requiring clear labeling that differentiates them from FDA-approved medications, and more.⁴⁹ Therefore, an important follow-up question is whether legislation restricting compounders could in fact have the opposite effect and instead drive hundreds of thousands of patients to switch from compounded to branded GLP-1s by year's end.

GLP-1 Market Expansion's Impact on Novo's Current and Pipeline Drugs

The final core question regarding NVO's future price action throughout the remainder of the year is whether clinical trial efficacy readouts from drugs in the rapidly proliferating GLP-1 market will cause NVO's own stock to experience non-operationally related volatility. Since the development of Ozempic and Wegovy as the dominant current-generation GLP-1 drugs for T2D and weight loss, the GLP-1 pipeline has expanded from a once-domineering duopoly to a

⁴⁹ Office of Senator Jim Banks, "Senate HELP Committee Advances Sen. Banks' SAFE Drugs Act," press release, July 23, 2026, <https://www.banks.senate.gov/news/press-releases/senate-help-committee-advances-sen-banks-safe-drugs-act/>.

fiercely competitive landscape with several players vying to topple the long-held duopolists' control on the fastest-growing sector in pharmaceutical history. Previously, the main blockades that smaller GLP-1 companies faced were the extensive infrastructure requirements and differentiated drug development that both Eli Lilly and Novo Nordisk have had years and abundant capital to develop.⁵⁰ However, as these GLP-1 producers have slowly built up their manufacturing capacity and built or bought their way to a differentiated drug portfolio, they have begun to pose a continuously increasing threat to the two market giants.

One company that now poses an especially noteworthy threat to both Novo Nordisk and Eli Lilly is Pfizer, which in November 2025 acquired next-generation drug producer Metsera after competing with Novo Nordisk in a bidding war that raised Pfizer's bid for the company from \$7.3 billion to \$10 billion.⁵¹ The Metsera acquisition offered two key product differentiations that made the company an important investment: extended drug half-life and exceptional patient tolerability. The two key drugs that were the focus points for both Pfizer and Novo Nordisk were Berobenatide (PF'3944), a Long-Acting injectable GLP-1 agonist with a 380-hour half-life, and PF'3945, a monthly amylin analog. In addition to it having the longest half-life of any peptide-based GLP-1, Berobenatide has shown a 14.1% placebo-subtracted weight-loss efficacy at just 28 weeks, as well as almost unseen patient tolerability, with only 2 out of 239 patients discontinuing the drug during a phase 2 study.⁵² PF'3945's value stems largely from its ability to potentially be combined with Berobenatide to form the first Nutrient

⁵⁰ GlobalData Healthcare, "GLP-1R Market: A Two-Horse Race," Pharmaceutical Technology, August 20, 2024, <https://www.pharmaceutical-technology.com/analyst-comment/glp-1r-market-two-horse-race/>.

⁵¹ Darren Incorvaia, "Pfizer Finalizes Metsera Buy after Contentious Bidding War with Novo Nordisk," Fierce Biotech, November 13, 2025, <https://www.fiercebiotech.com/biotech/pfizer-finalizes-metsera-buy-after-contentious-bidding-war-novo-nordisk>.

⁵² James Waldron, "Pfizer's New Prize METSERA Touts 14% Weight Loss, 'class-Leading Tolerability' for Lead GLP-1 Drug," Fierce Biotech, September 30, 2025, <https://www.fiercebiotech.com/biotech/pfizers-new-prize-metsera-touts-141-weight-loss-lead-glp-1-drug>.

Stimulated Hormone (NuSH) combination drug, joining the GLP-1 and amylin drugs to induce an expected high teens weight-loss efficacy and once-monthly administration requirement.⁵³

Furthermore, while they may not have the extent of manufacturing capacity that Novo Nordisk, Eli Lilly, and Pfizer have, there are several other companies developing their own GLP-1 competitors. Amgen's [MariTide](#) has shown up to 13.7% placebo-adjusted weight loss after 52 weeks in patients with obesity without reaching a weight loss plateau and has the potential for monthly or even less frequent dosing.⁵⁴ Roche's [Enicepatide](#) dual agonist has shown an 18.8% weight loss at 24 weeks after adjusting for placebo efficacy, with hopes that it could show 25% weight-loss efficacy in later stage trials.⁵⁵ As noted before, innovation exists in the oral space as well with Structure Therapeutics' Aleniglipron showing 6.2% placebo-adjusted weight-loss efficacy after just 12 weeks in clinical trials and presenting itself as a possible alternative to Oral Wegovy and Foundayo.⁵⁶

Given that most of these drugs are still in Phase 1 through Phase 3 clinical trials, they will not be FDA-approved and commercially available by the end of 2026, even if they were to receive a Commissioner's National Priority Voucher (CNPV) like Foundayo. However, although the TRx outcome will largely be determined by consumers evaluating each drug's restrictions, a more impactful event than competitors' drugs' FDA approval could very well be their clinical

⁵³ Metsera Inc., "Metsera Pipeline: Next-Generation Therapies," Metsera, 2025, <https://metsera.com/pipeline/>.

⁵⁴ Elissa Snook and Justin Claeys, "Amgen Announces Robust Weight Loss with Maritide in People Living with Obesity or Overweight at 52 Weeks in a Phase 2 Study," Amgen, November 26, 2024, <https://www.amgen.com/newsroom/press-releases/2024/11/amgen-announces-robust-weight-loss-with-maritide-in-people-living-with-obesity-or-overweight-at-52-weeks-in-a-phase-2-study>.

⁵⁵ James Waldron, "'The Trajectory Is Encouraging.' Roche Hopes Obesity Drug Will Show 25% Weight Loss One Day," Fierce Biotech, September 11, 2024, <https://www.fiercebiotech.com/biotech/trajectory-encouraging-roche-hopes-obesity-drug-will-show-25-weight-loss-one-day>.

⁵⁶ Structure Therapeutics, "Structure Therapeutics Reports Positive Topline Data from Its Phase 2A Obesity Study and Capsule to Tablet PK Study for Its Oral Non-Peptide Small Molecule GLP-1 Receptor Agonist GSBR-1290," Structure Therapeutics, June 3, 2024, <https://ir.structuretx.com/news-releases/news-release-details/structure-therapeutics-reports-positive-topline-data-its-phase>.

trial efficacy readouts. On this front, there exist several key readouts expected by year-end including Boehringer Ingelheim and Zealand Pharma's weight-loss and MASH drug [Survodutide](#), as well as two oral competitors with Structure Therapeutics' Aleniglipron and Viking Therapeutics' VK2735. Each of these drugs has expected efficacy trial readouts before year-end 2026, including Survodutide's SYNCHRONIZE-2 obesity and T2D trial, Aleniglipron's ACCESS OLE weight-loss durability trial, and VK2735's VANQUISH reduced dosing frequency weight-maintenance efficacy sub-study. Nevertheless, while NVO investors have historically reacted strongly to its own drug's efficacy readouts, they have had varying reactions to positive or negative efficacy readouts from Eli Lilly's own drugs.

The most notable example of this is the Zepbound/tirzepatide SURMOUNT-5 efficacy trial readout, where Zepbound was compared to Wegovy in weight loss over a 72-week period. Following the release of initial top-line results from the head-to-head trial on December 4, 2024, Eli Lilly's stock saw a small appreciation of 2% while NVO dropped just 0.2%. This trend continues when comparing the price action for other core trial readouts, including the TRIUMPH-4 Retatrutide efficacy trial that outlined up to 28.7% weight-loss efficacy and the ATTAIN-1 Orforglipron (Foundayo) efficacy trial top-line readout. For the former trial, Eli Lilly's stock rose roughly 1.5% on December 11, 2025, the top-line release day, and 5% in the two trading days following its release while NVO's share price rose day-of by 2.5% and maintained those gains in the two trading days following the top-line release. For the latter trial, there did exist a strong correlation trend however, with Eli Lilly's stock rising roughly 7.5% on August 7, 2025, the top-line release day, and growing 2.5% in the two trading days following while NVO experienced a 14% drop day-of and a roughly 1% further drop in the following two trading days.

Given these varying trends in reactions to external drugs' efficacy readouts, the listed clinical trials and their readouts pose the possibility for extreme reactions like what was shown with the Orforglipron ATTAIN-1 top-line readout or muted reaction like the Retatrutide TRIUMPH-4 efficacy readout. While I don't have enough insight to determine which pose especially significant threats to NVO's share price, I do believe the only notable outcomes would cause downwards pressure on the stock. This assertion assumes that current valuations likely already include a preconceived notion of Oral Wegovy's market share retention capabilities without much inclusion of oral competitors given the early stage they are in. Therefore, a positive readout would primarily serve to provide downward pressure on NVO's price while a negative readout would likely have minimal impact given the clinical trial stage many oral competitors are currently in.

Conclusion:

In summary, Novo Nordisk is a company that has suffered severe market share loss due to superior competitor drug efficacy. While it suffered a recent 14% decrease in its share price following the negative ZEUS trial Ziltivekimab readout and poor forward guidance, I argue there exists meaningful room for further price depreciation as consumers adapt to the new GLP-1 landscape of oral weight-loss and T2D alternatives. While the stock remains near 5-year lows of \$35.12, which it achieved in late March of 2026, I argue the stock remains actionable for further share price depreciation by the end of 2026.

My core thesis for NVO's share price decreasing by year-end relies on three components that are coalescing in this year's remaining two quarters: Foundayo gaining strong marketing backing after establishing prescriber relationships and distribution channels, consumers

prioritizing long-term tolerability and lifestyle integration over top-line efficacy with oral drugs specifically, and current decreasing TRx growth rates for Oral Wegovy serving as a foundation for slowing consumer demand and a prime opportunity for Foundayo to cannibalize Oral Wegovy's currently domineering US oral GLP-1 market share. Finally, while there remain several key risks and possibilities for unpredictable price volatility to occur in the remaining half of the year, Novo's current share price valuation relies heavily on Oral Wegovy's continued performance as the leading oral weight-loss drug following Ziltivekimab's efficacy failure. Therefore, I argue that while Oral Wegovy does have superior weight-loss efficacy over Foundayo, the efficacy-fueled decision matrix for consumers will continue to hold less prominence, with consumers instead focusing on one key factor instead. It is the key factor that I argue Foundayo is superior in and off which the drug will ultimately cannibalize a significant portion of Oral Wegovy's US market share: Convenience.

Appendix

Novo Nordisk and Competitor Drugs' Clinical Trial Results

Drug	Target Population	Efficacy	Tolerability	Administration
<u>Oral Wegovy</u> (semaglutide) (Novo Nordisk) (Commercially Available)	Chronic Weight Management (CWM) for Obese or Overweight with one or more weight-related comorbidities, MACE Risk Reduction. Not pursuing T2D approval since Rybelsus exists	Weight loss: -25 mg, 64 weeks: 13.6% versus 2.2% placebo Treatment Policy Estimand [TP] (OASIS-4)	GI event frequency: -25 mg, 64 weeks: 74% oral versus 42% placebo (OASIS-4) Severe GI event frequency: -2% versus 0% placebo (United States Prescribing Information, USPI) Discon. due to adverse reactions: -6.9% for 25 mg versus 5.9% placebo (OASIS-4) Discon. due to GI: -3.4% oral versus 2% placebo (OASIS-4)	Once-daily Oral Pill: First thing in the morning, empty stomach, ≤4 oz water, wait ≥30 min (large-molecule peptide analog structure) *The fasting and water restrictions are driven by SNAC-mediated absorption stemming from the pill's molecular structure.
<u>Foundayo</u> (Orforglipron) (Eli Lilly) (Commercially Available)	CWM for Obese or Overweight with one or more weight-related comorbidities, looking into T2D approval	Weight loss: -5.5 mg, 9 mg, 17.2 mg, 72 weeks: 7.4%, 8.3%, 11.2% versus 2.1% placebo [TP] (ATTAIN-1) T2D: -5.5 mg, 9 mg, 17.2 mg, 72 weeks: 1.2%, 1.4%, 1.7% versus 0.4% placebo A1C reduction [TP] (ATTAIN-2)	GI event frequency: -60% for 5.5 mg, 68% for 9 mg, 69% for 17.2 mg versus 37% for placebo (ATTAIN-1+ATTAIN-2) Discon. due to adverse reactions: -6% for 5.5 mg, 9% for 9mg, 10% for 17.2 mg versus 3% placebo (ATTAIN-1+ATTAIN-2) Discon. due to GI: -3% for 5.5 mg, 6% for 9 mg, 6% for 17.2 mg versus 0.7% placebo (ATTAIN-1+ATTAIN-2)	Once-daily Oral Pill: any time of the day, no food or water restriction (small-molecule non-peptide structure) *Label-level restrictions absent from Oral Wegovy: maximum dose of 9 mg with a strong CYP3A4 inhibitor, simvastatin capped at 20 mg (since Foundayo roughly doubles blood levels of simvastatin), oral hormonal contraceptives require a switch to a non-oral method (ring, implant, IUD) or an added barrier method for 30 days after initiation and after each dose escalation

Wegovy (semaglutide) (Novo Nordisk) (Commercially Available)	CWM for Obese or Overweight with one or more weight-related comorbidities, and MASH liver disease treatment (F2-F3 Fibrosis), MACE (Major Adverse Cardiovascular Events) risk reduction	Weight Loss: -7.2 mg, 2.4 mg, 72 weeks: 20.7%, 17.5%, 2.4% Efficacy Estimand [Eff] (STEP UP) -7.2 mg, 2.4 mg, 72 weeks: 18.7%, 15.6%, 3.9% [TP] (STEP UP) MASH: -Fibrosis improvement without worsening steatohepatitis: 37% versus 22.5% placebo [n/d] (ESSENCE) -2.4 mg, 240 weeks: MASH resolution without worsening fibrosis: 62.9% versus 34.1% placebo [n/d] (ESSENCE) MACE: - 2.4 mg, up to 5 years: 23% risk reduction (6.5% versus 8% placebo occurrence of MACE) (SELECT)	GI event frequency: -70.9% for 7.2 mg, 61.2% for 2.4 mg versus 42.8% placebo (STEP UP) Severe GI event frequency: -4.1% versus 0.9% placebo (USPI) Discon. due to adverse reactions: -5% for 2.4 mg and 7.2 mg versus 2% placebo (STEP UP + STEP UP T2D) Discon. due to GI: -4.3% for Wegovy injection versus 0.7% placebo (USPI)	Once-weekly Subcutaneous Injection, no dietary or timing restrictions
Zepbound (tirzepatide) (Eli Lilly) (Commercially Available)	CWM for Obese or Overweight with one or more weight-related comorbidities, Obstructive Sleep Apnea (OSA) with Obesity	Weight Loss: -5 mg, 10 mg, 15 mg, 72 weeks: 15%, 19.5%, 20.9% versus 3.1% placebo (SURMOUNT-1) -15 mg, 84 weeks: 23.6% versus N/A placebo (REDEFINE 4)	GI event frequency: -56% for 5 mg, 56% for 10 mg, 56% for 15 mg versus 30% placebo (USPI) Severe GI event frequency: -1.7% for 5 mg, 2.5% for 10 mg, 3.1% for 15 mg versus 1% placebo Discon. due to GI adverse reactions: -1.9% for 5 mg, 3.3% for 10 mg, 4.3% for 15 mg versus 0.5% placebo (USPI)	Once-weekly Subcutaneous Injection (Auto-inject Pen or Vial), No dietary or timing restrictions
Ozempic (semaglutide) (Novo Nordisk) (Commercially Available)	Improving blood sugar control for patients with T2D, CV risk reduction, and reducing the risk of worsening kidney disease	T2D: -0.5 mg, 56 weeks: 1.32% A1C reduction (SUSTAIN-2) -1 mg, 56 weeks: 1.57 % A1C reduction (SUSTAIN-2 + -3) -1 mg, 40 weeks: 1.86% A1C reduction (SURPASS 2) -2 mg, 40 weeks: 2.2% A1C reduction (SUSTAIN FORTE)	GI event frequency: -32.7% for 0.5 mg, 36.4% for 1 mg, 15.4% for placebo (USPI) -34% for 2 mg, 30.8% for 1 mg versus N/A placebo (USPI) Severe GI event frequency: -0.4% for 0.5 mg, 0.8% for 1 mg versus 0% placebo (USPI) Discon. due to GI adverse reactions: -3.1% for 0.5 mg, 3.8% for 1 mg versus 0.4% placebo (USPI)	Once-weekly Subcutaneous Injection, no dietary or timing restrictions

Mounjaro (tirzepatide) (Eli Lilly) (Commercially Available)	Improving blood sugar control for patients with T2D	T2D: -5 mg, 40 weeks: 2.01% A1c reduction (SURPASS 2) -5 mg, 104 weeks: 2.3% A1C reduction (SURPASS-4) -10 mg, 40 weeks: 2.24% A1C reduction (SURPASS 2) -10 mg, 104 weeks: 2.5% A1C reduction (SURPASS-4) -15 mg, 40 weeks: 2.30% A1C reduction (SURPASS 2) -15 mg, 104 weeks: 2.6% A1C reduction (SURPASS-4)	GI event frequency: -37.1% for 5 mg, 39.6% for 10 mg, 43.6% for 15 mg versus 20.4% placebo (USPI) Severe GI event frequency: -1.3% for 5 mg, 0.4% for 10 mg, 1.2% for 15 mg versus 0.9% placebo Discon. due to GI adverse reactions: -3% for 5 mg, 5.4% for 10 mg, 6.6% for 15 mg versus 0.4% placebo (USPI)	Once-weekly Injection, no dietary or timing restrictions
Rybelsus/Ozempic Pill (semaglutide rebrand, ~ same product): 3 mg, 7 mg, and 14 mg are pharmacologically equivalent to the new 1.5 mg, 4 mg, and 9 mg for the Ozempic Pill (Novo Nordisk) (Commercially Available)	Improving blood sugar control for patients with T2D	T2D: -14 mg (new 9 mg), 52 weeks: 1% - 1.5% A1C reduction (PIONEER-4 + -8) -7 mg (new 4 mg), 26 weeks: 1.0% A1C reduction (PIONEER-3)	GI event frequency: -41% for 14 mg, 32% for 7 mg versus placebo 21% Severe GI event frequency: -2.0% for 14 mg, 0.6% for 7 mg, versus 0.3% placebo Discon. due to adverse reactions: -8% for 14 mg, 4% for 7 mg versus 1% placebo	Once-daily Oral Pill
Saxenda (liraglutide) (Novo Nordisk) (Commercially Available)	CWM for Obese or Overweight with one or more weight-related comorbidities	Weight Loss: -3 mg, 56 weeks: 7.4% versus 3% placebo (estimand not defined [n/d]) (USPI)	GI event frequency: -68% versus 39% placebo (dose not outlined) (USPI) Severe GI event frequency: -4.8% versus 1.4% placebo (USPI) Discon. due to adverse reactions: -9.8% versus 4.3% placebo (USPI)	Once-daily Subcutaneous Injection
Byetta (Exenatide) (Amylin Pharmaceuticals) (Discontinued)	Improving blood sugar control for patients with T2D	T2D: -5 mcg, 10 mcg, 24 weeks: 0.7%, 0.9% versus 0.2% A1C reduction (USPI)	GI event frequency: -4% versus 0% placebo Severe GI event frequency: -0% Discon. due to adverse reactions: -1.3% versus 0% placebo	Twice-Daily Subcutaneous Injection; injections should happen within 60 minutes before the morning and evening meals and cannot be administered after a meal.

Trulicity (Dulaglutide) (Eli Lilly) (Commercially Available)	Improving blood sugar control for patients with T2D, ASCVD / CV Risk Reduction in T2D patients	T2D: 0.75 mg, 1.5 mg, 26 weeks: 0.7%, 0.8% versus 0.6% ~placebo (USPI)	GI event frequency: -31.6% for 0.75 mg, 41% for 1.5 mg versus 21.3% for placebo Severe GI event frequency: -7% for 0.75 mg, 11% for 1.5 mg Discon. due to adverse reactions: -1.3% for 0.75 mg, 3.5% for 1.5 mg versus 0.2% placebo	Once-weekly Subcutaneous Injection
Victoza (liraglutide) (Novo Nordisk) (Commercially Available)	Improving blood sugar control for patients with T2D, CV risk reduction	T2D: -1.2 mg, 1.8 mg, 52 weeks: 0.8%, 1.1% A1C reduction (USPI)	GI event frequency: - Discon. due to GI AE: 4.3% vs 0.5% placebo (USPI) - GI event rate and severe GI rate: n/d	Once-daily Subcutaneous Injection
CagriSema (semaglutide + Cagrilintide) (Novo Nordisk) (Awaiting FDA Approval)	CWM for Obese or Overweight with one or more weight-related comorbidities	Weight Loss: 2.4 mg/2.4 mg, 84 weeks: 20.2% Trial policy estimand or 23% Efficacy estimand versus N/A placebo (REDEFINE-4) 2.4/2.4, 68 weeks, 20.4% versus 3% placebo (REDEFINE-1)	GI event frequency: -79.4% for 2.4/2.4 mg versus 39.9% placebo (REDEFINE-1) Discon. due to adverse reactions: -5.9% for 2.4/2.4 mg versus 3.5% placebo (REDEFINE-1) -8.4% for 2.4/2.4 mg versus 3% placebo (REDEFINE-2)	Once Weekly fixed-dose SC
Retatrutide (Retatrutide) (Eli Lilly) (Phase 3)	CWM for Obese or Overweight with one or more weight-related comorbidities, Overweight with T2D, CVD, Knee Osteoarthritis pain, Obstructive sleep apnea (OSA), chronic back pain, Metabolic Dysfunction-Associated Steatotic Liver Disease (MASLD)	Weight loss: 80 Weeks, 4 mg, 9 mg, 12 mg: 19.0%, 25.9%, 28.3% vs 2.2% placebo (TRIUMPH-1) 68 weeks, 12 mg: 28.7% versus 2.1% placebo (TRIUMPH-4) T2D: HbA1c Reduction: 40 weeks, 4 mg, 9 mg, 12 mg: -1.69%, -1.86%, -1.94% vs -0.81% placebo	GI event frequency: -22.1% for 4 mg, 30.3% for 9 mg, 31.5% for 12 mg versus 11% placebo (mean of 4 GI adverse reaction listed in TRIUMPH-1 news release) Discon. due to adverse reactions: -4.1% for 4 mg, 6.9% for 9 mg, 11.3% for 12 mg versus 4.9% placebo (TRIUMPH-1)	Once-weekly SC Triple agonist: GIP, GLP-1, Glucagon
Berobenatide (PF'3944 / MET-097i) (Pfizer) (Phase 3)	CWM for Obese or Overweight with one or more weight-related comorbidities	Weight loss: 1.2 mg, 28 weeks: 14.1% PA (Phase 2b VESPER-1) (Weekly dosing) 28 weeks: 12.3% PA (Phase 2b Vesper-3) (Monthly dosing after week 12)	Discon. due to adverse reactions: Market-leading tolerability: 2 out of 239 (0.8%) patients discontinued the drug in a phase 2 study (VESPER-1)	Subcutaneous, once-weekly, Subcutaneous, once-monthly

		2.4 mg, 32 weeks: 15.9% versus N/A placebo (VESPER 1, weekly dosing T2D: 1.6 mg, 28 weeks: 2.2% A1C reduction versus 0.2% placebo (Treatment efficacy estimand, VESPER- 2)		
MariTide (maridebart cafraglutide) (Amgen) (Phase 3)	CWM for Obese or Overweight with one or more weight-related comorbidities, Improving blood sugar control for patients with T2D	Weight loss: 140 mg – 420 mg, 52 weeks: 12.3%-16.2% versus 2.5% placebo (NCT05669599) T2D: 52 weeks, 140 – 420 mg: 1.2% - 1.6% reduction in A1C (NCT05669599)	N/A	Subcutaneous, once-weekly
Enicepatide (CT- 388) (Roche) (Phase 3)	CWM for Obese or Overweight with one or more weight-related comorbidities	Weight loss: 24 mg, 48 weeks: 22.5% placebo- adjusted (PA) efficacy estimand with no plateau (CT388-103) T2D: not significant enough data	-N/A	Subcutaneous, once-weekly
Zenagamtide (Amycretin) (Novo Nordisk) (Phase 3)	CWM for Obese or Overweight with one or more weight-related comorbidities, Improving blood sugar control for patients with T2D	Weight loss: (SC) 60 mg, 20 mg, 36 weeks: 23.2% PA, 20.1% PA 5 mg, 28 weeks: 13.9% PA 1.25 mg, 20 weeks: 7.7% PA (NCT06064006) T2D: (SC) 40 mg, 20 mg, 10 mg, 5 mg, 36 weeks: 1.71%, 1.64%, 1.41%, 1% versus 0.14% placebo A1C reduction (Phase 2 diabetes trial)	N/A	Subcutaneous, once-weekly, Oral, once-daily
Survodutide (Boehringer Ingelheim + Zealand Pharma) (Phase 3)	CWM for Obese or Overweight with one or more weight-related comorbidities, MASH Liver Disease treatment	Weight loss: 76 weeks: 16.6% versus 3.2% placebo (SYNCHRONIZE-1) MASH: 2.4 mg, 4.8 mg, 6 mg, 48 weeks: 34%, 36%, 34% versus 22% placebo improvement in fibrosis by at least	-N/A	Subcutaneous, once-weekly

		one stage (Phase 2 MASH Trial) 6 mg 76 weeks: 63.1% reduction in liver fat, 34% reduction in visceral fat (SYNCHRONIZE-1)		
VK2735 (Viking Therapeutics) (Phase 3)	CWM for Obese or Overweight with one or more weight-related comorbidities	Weight loss: Oral 120 mg, 90 mg, 60 mg, 13 weeks: 12.2%, 11.1%, 8.7% versus 1.3% placebo SC 17.5 mg, 13 weeks: 14.7% versus N/A placebo, no signs of plateau	Discon. due to adverse reactions: -28% versus 18% placebo	Subcutaneous, once-weekly, Oral, once-daily
Pemvidutide (Altimune) (Transitioning into Phase 3)	CWM for Obese or Overweight with one or more weight-related comorbidities, MASH Liver Disease treatment	Weight Loss: 1.2 mg, 1.8 mg, 24 weeks: 5%, 6.2%, versus 1% placebo (IMPACT Phase 2 Trial) 1.2 mg, 1.8 mg, 48 weeks: 4.5%, 7.5%, versus 0.2% placebo (IMPACT Phase 2 Trial) MASH: 1.2 mg, 1.8 mg, 24 weeks: 58%, 62.8% versus 16.2% placebo reduction of liver fat (IMPACT Phase 2 Trial) 1.2 mg, 1.8 mg, 48 weeks: 45.2%, 54.7% versus 8.2% placebo reduction of liver fat (IMPACT Phase 2 Trial)	Discon. due to adverse reactions: -0% (1.2 mg) and 1.2% (1.8 mg) versus 2.4% placebo at 24 weeks -0% (1.2 mg) and 1.2% (1.8 mg) versus 3.5% placebo at 48 weeks	Subcutaneous, once-weekly
Petrelintide (Zealand Pharma + Roche / Genentech) (Transitioning into Phase 3)	CWM for Obese or Overweight with one or more weight-related comorbidities	Weight loss: Max dose, 42 weeks: 10.7% versus 1.7% placebo (efficacy estimand)	Discon. due to adverse reactions: -4.8% versus 4.9% placebo	Subcutaneous, once-weekly

Aleniglipron (Structure Therapeutics) (Transitioning into Phase 3)	CWM for Obese or Overweight with one or more weight-related comorbidities	Weight loss: 120 mg, 240 mg, 36 weeks: 11.3% PA, 15.3% PA (ACCESS I-II) 44 weeks, 120 mg, 240 mg: 14.7% PA, 16.0% PA no plateau (ACCESS II)	Discon. due to adverse reactions: Mean 10.4% across all ACCESS I arms	Small molecule nonpeptide Oral, once-daily
Elecoglipron (AstraZeneca) (Transitioning into Phase 3)	CWM for Obese or Overweight with one or more weight-related comorbidities, Improving blood sugar control for patients with T2D	Weight loss: 5 mg, 15 mg, 50 mg, 75 mg, 26 weeks: 2.6%, 5.6%, 8.1%, 10% versus 0.6% placebo (VISTA) 5 mg, 15 mg, 50 mg, 75 mg, 36 weeks: 2.7%, 6.5%, 9.2%, 11.8% versus 0.3% placebo (VISTA) T2D: 5 mg, 75 mg, 26 weeks: 0.91% A1C reduction, 1.88% A1C reduction versus 0.15% placebo (SOLSTICE)	Discon. due to adverse reactions: -6% across all drug doses versus 4% in placebo (VISTA)	once daily oral small-molecule glucagon-like peptide-1 (GLP-1) receptor agonist
PF-3945 (MET-233i) (Pfizer) (Phase 2)	CWM for Obese or Overweight with one or more weight-related comorbidities	Weight loss: N/A	N/A	-Once monthly (target) GLP-1 + Amylin combo SC -Possible combination with Berobenatide to create first Nutrient Stimulated Hormone (NuSH)
UBT251 (Novo Nordisk) (Phase 1b/2a)	CWM for Obese or Overweight with one or more weight-related comorbidities, Improving blood sugar control for patients with T2D	Weight loss: 24 weeks: 19.7% versus 2% placebo (Phase 2 obese population trial) T2D: 2.16% A1C reduction (Phase 2 T2D population trial)	Discon. due to adverse reactions: -0% AE-driven discontinuations at top doses (Chinese Phase 1b trial)	Once-weekly SC Triple agonist: GIP, GLP-1, Glucagon
Xinermei (Mazdutide) (Innovent Biologics + Eli Lilly)	CWM for Obese or Overweight with one or more weight-related comorbidities, Improving blood sugar control for patients with T2D, MASH Liver Disease treatment, Obstructive sleep apnea (OSA)	Weight loss: 4 mg, 6 mg, 48 weeks: 12%, 14.8%, versus 0.5% placebo (GLORY-1) 9 mg, 60 weeks: 20.1% versus 2.8% placebo MASH: 4 mg, 6 mg, 48 weeks: 65.85%, 80.24% versus 5.27% placebo reduction of liver fat (GLORY-1)	Discon. due to adverse reactions: -2.9% (9 mg) versus 0% placebo	Subcutaneous, once-weekly

		6 mg 76 weeks: 34% reduction in visceral fat at only a 11.3% change in total tissue mass (GLORY)		
Hemlibra (Emicizumab) (Genentech/Roche) (Commercially Available)	Hemophilia A	Estimated Mean Annualized Bleeding Rate (ABR): Pooled median ABR: 0.5	Minimal adverse events, with no life-threatening bleeds or thrombotic complications. The presence of non-neutralizing anti-drug antibodies in one patient did not result in clinical efficacy loss, indicating the rarity and limited clinical impact of such occurrences.	Subcutaneous, once-weekly, Subcutaneous, once-biweekly, Subcutaneous, once-monthly
Fitusiran (Qfitia) (Sanofi) (Commercially Available)	Hemophilia A (reduces antithrombin rather than infusing Factor VIII or IX)	Estimated Mean Annualized Bleeding Rate (ABR): Population with no Factor VIII inhibitors, Population with Factor VIII inhibitors, 9 months: 71% ABR reductions to 9.0 vs 31.4 treatment with clotting factor concentrates, 73% ABR reduction with 5.1 versus 19.1 for those treated with an on-demand bypassing agent (Phase 3 ATLAS)	Potentially serious adverse reactions, including thrombotic events, acute and recurrent gallbladder disease, and hepatotoxicity. An additional warning noted liver toxicity and the need to monitor liver blood tests at baseline and monthly for at least 6 months after initiating therapy. The most commonly reported side effects—occurring in more than 10% of patients—were viral infections, nasopharyngitis, and bacterial infections. (Phase 3 ATLAS)	Subcutaneous, once-bimonthly
Denecimig (Mim8) (Novo Nordisk) (Awaiting FDA Approval)	Hemophilia A	Estimated Mean Annualized Bleeding Rate (ABR): Once-weekly, Once-monthly, 26 weeks: 0.57 (-97%), 0.2 (-99%) versus 15.8 for patients taking no clotting factor concentrates (FVIII) Once-weekly, Once-monthly, 26 weeks: 2.5 (-48%) versus 4.8 patients who had been receiving FVIII replacements during	No patient was reported to have a thromboembolic event or clinical evidence of neutralizing anti-Mim8 antibodies.	Subcutaneous, once-weekly, Subcutaneous, once-bi-weekly, Subcutaneous, once-monthly

		a run-in phase, 1.8 (-43%) versus 3.1 Patients who had been receiving FVIII replacements during a run-in phase		
Zemocimig (NXT007) (Chugai/Roche) (Transitioning into Phase 3)	Hemophilia A	Estimated Mean Annualized Bleeding Rate (ABR): 0.072 mg, 0.28 mg, 0.70 mg, 1.08 mg, various times: 1.48, 0.28, 0, 0 (Extremely small test sizes, 10, 6, 6, 8)	NXT007 had a favorable safety profile at all doses. Most AEs were mild/moderate, and all 3 SAEs were considered unrelated to NXT007.	Subcutaneous, once-monthly
Rezdiffra (Resmetirom) (Madrigal) (Commercially Available)	MASH Liver Disease Treatment (F2-F3)	MASH: 80 mg, 100 mg, 52 weeks: 23%, 26% versus 14% placebo of patients with at least one stage improvement in liver fibrosis and no worsening of steatohepatitis (USPI, mean of two pathologists' analyses)	Discon. due to adverse reactions: -5% for 80 mg, 8% for 100 mg versus 4% for placebo (USPI)	Oral, once-daily
Efruxifermin (Novo Nordisk) (Phase 3)	MASH Liver Disease Treatment (F2-F3 and F4 compensated Cirrhosis)	MASH: 28 mg, 50 mg, 96 weeks: 46%, 75% versus 24% placebo improvement in fibrosis by at least one stage for F2-F3 patients (Phase 2b HARMONY) 28 mg, 50 mg, 36 (96) weeks: 18% (21%), 19% (29%) versus 13% (11%) placebo reduction in fibrosis without worsening MASH in F4 cirrhosis patients (Phase 2b SYMMETRY)	38 (95%) of 40 participants in the 28 mg group, 43 (100%) of 43 in the 50 mg group, and 42 (98%) of 43 in the placebo group reported an adverse event. (Phase 2b HARMONY)	Subcutaneous, once-weekly

Pegozafermin (Roche) (Phase 3)	MASH Liver Disease Treatment (F2-F3 and F4 compensated Cirrhosis), Severe Hypertriglyceridemia (SHTG)	MASH: 15 mg, 30 mg, 44mg, 24 weeks: 22%, 26%, 27% versus 7% placebo of patients with at least one stage improvement in liver fibrosis and no worsening of NASH (MASH) (Phase 2b ENLIVEN)	Discon. due to adverse reactions: -5% for 15 mg, 6% for 30 mg, 2% for 44 mg versus 0% for placebo (Phase 2b ENLIVEN)	Subcutaneous, once-weekly, Subcutaneous, once-biweekly
Efimosfermin (GSK) (Phase 3)	MASH Liver Disease Treatment (F2-F3 and F4 compensated Cirrhosis)	MASH: 89% of participants had at least a 30% reduction in hepatic fat	Of 65 participants treated with Efimosfermin, 43 (66%) had treatment-emergent adverse events (TEAEs)	Subcutaneous, once-biweekly Subcutaneous, once-monthly
Ziltivekimab (Novo Nordisk) (Phase 3)	MACE Reduction in ASCVD, CKD, and inflammation population	MACE risk reduction: 7.5 mg, 15 mg, 30 mg, 12 weeks: -77%, -88%, -92% versus -4% placebo median change from baseline hsCRP (Phase 2 trial)	N/A	Subcutaneous, once-monthly

Novo Nordisk and Competitor Drugs' Market Positioning

Drug	TRx Share	Net Price GTN	Retention	Recent Milestones
Oral Wegovy (semaglutide) (Novo Nordisk) (Commercially Available)	Peak ~160,000 TRx (early June 2026), ~145,000 on July 2, 2026, with slowdown trend	List price of \$1,349, ~200 blended net price, meaning ~85% GTN, self-pay is largely the dominant channel	N/A, oral market still in early stages pre Foundayo ramp	December 2025: US FDA approval of Oral Wegovy as the first-ever weight-loss oral GLP-1 drug January 2026: US launch of Oral Wegovy August 2026: Softer-than-expected sales growth for Oral Wegovy as TRx shows slowing prescription growth.

Foundayo (Orforglipron) (Eli Lilly) (Commercially Available)	~20,000 TRx (Late June 2026) with stagnating growth due to key factors: Manufacturing limits, T2D approval wait, Medicare channel not included in numbers, measurement inconsistencies in telehealth prescription numbers, no quarterly results for Foundayo yet	N/A	N/A, oral market still in early stages pre Foundayo ramp	End of Q2: Planned FDA submission of Foundayo for T2D treatment June 2026: ACHIEVE 2,3, and 5 data: comparing Foundayo 17.2 mg to Oral Wegovy 14 mg April 2026: US Commercial launch
SC Wegovy (semaglutide) (Novo Nordisk) (Commercially Available)	Out of the Wegovy/Zepbound/Foundayo TRx weight-loss market: Total TRx hovers near 1,170,000 as of July 3, 2026, Wegovy Pen (all SC), holds about 320,000, or about 27.4% of the market For Wegovy's MASH capabilities, Wegovy's share of the TRx market for Wegovy and Rezdiffra issued by Hepatologists (Liver-focus) has shifted from ~25% to ~65% from August 2025 to July 3 2026	As of Q1 2026, Gross-to-net hovers near -65% for SC Wegovy (all Wegovy injections)	One-year persistence by start date: Q1 2024: 62.7% 1-to-3-year persistence by start date: 2021-Q12022: 33% à 20% à 14.3% (Best persistence of any drug measured)	July 2026: UC San Diego School of Medicine reports results from the first clinical trial to demonstrate that semaglutide may improve liver fibrosis in patients with advanced, F4 compensated cirrhosis, possibly bolstering its F4 MASH portfolio that currently consists principally of Efruxifermin. March 2026: FDA US approval of Wegovy HD for previous 2.4 mg users 2026: semaglutide LOE in large countries including India, Canada, China, Brazil, and Turkey
Zepbound (tirzepatide) (Eli Lilly) (Commercially Available)	Out of the Wegovy/Zepbound/Foundayo TRx weight-loss market: Total TRx hovers near 1,170,000 as of July 3, 2026, Zepbound Pen hovers near 475,000 while the Zepbound Vial hovers near 215,000, composing 40.6% and 18.4% of the market, respectively	As of Q1 2026, Gross-to-net hovers near -35% for Zepbound Pen and -50% for Zepbound Vial	One-year persistence by start date: Q1 2024: 62.4%	February 2026: FDA approval of Zepbound to include the four-dose, single-patient use KwikPen, allowing for a full month of treatment in a single multi-dose pen May 2025: SURMOUNT-5 Trial results announced comparing max dose Zepbound to Wegovy

Ozempic (semaglutide) (Novo Nordisk) (Commercially Available)	Shown in Figure 1.2, US and Global Diabetes GLP-1 Market Share has reverted to roughly the same percentages they were in mid-2019, with the entrance of Mounjaro in mid-to-late 2022, decreasing value market share from a peak of near 58% to the current 46% for both groups. TRx on a L12W basis has remained relatively stable, however, fluctuating around 520,000 from July 2024 to July 2026. It has given up parts of its US diabetes GLP-1 NBRx market share over the same period, shrinking from 50% to 38%.	Gross-to-net has lowered steadily each quarter, shifting from -28% in Q1 2020 to -60% in Q1 2026 for Ozempic	Based on a study analyzing Proportion of Days Covered (PDC, a measure of how many days in a specific period a patient has access to their prescribed medication, for T2D patients using semaglutide, retention remained strong at 83.9% between 0-6 months and 74.6% between 18-24 months. Another Study comparing medication persistence between oral and subcutaneous semaglutide saw retention of 94.8%, 92.4%, and 83.8% at 6, 12, and 18 months for subcutaneous semaglutide, respectively.	N/A
Mounjaro (tirzepatide) (Eli Lilly) (Commercially Available)	TRx for Mounjaro has steadily climbed higher over the past 2 years, rising from 350,000 to 850,000 from July 2024 to July 2026, respectively. The drug also holds the lion's share of the US diabetes GLP-1 NBRx market, having grown it from 32% to 55% over the same period.	N/A	Data is very noisy for Mounjaro/tirzepatide for T2D population. One study says it had a persistence rate (45-day-gap) of 73.3% at 6 months, meaning 73.3% of people didn't take a 45-day gap or longer during the 6-month analysis period.	February 2026: Combined sales of Mounjaro and Zepbound make it tirzepatide the world's best-selling drug, surpassing semaglutide (Ozempic and Wegovy) December 2025: The FDA expanded Mounjaro's approval to include pediatric patients ages 10 and older with type 2 diabetes.
Rybelsus/Ozempic Pill (semaglutide rebrand, ~ same product): 3 mg, 7 mg, and 14 mg are pharmacologically equivalent to the new 1.5 mg, 4 mg, and 9 mg for the Ozempic Pill (Novo Nordisk) (Commercially Available)	TRx on a L12W basis has remained extremely stable over the past two years, barely moving from around 55,000 TRx from July 2024 to July 2026.	Gross-to-Net has trended lower from -39% in Q1 2020 to -61% in Q1 2026 for Rybelsus.	A study comparing medication persistence between oral and subcutaneous semaglutide saw retention of 85.3%, 72.3%, and 46.0% at 6, 12, and 18 months for oral semaglutide, respectively.	February-May 2026: Rybelsus Oral semaglutide rebrand to Ozempic pill, with new dose sizes that provide the same effects as their respective previous dose sizes under Rybelsus, the rebrand was entirely driven by the Ozempic brand name recognition and relative obscurity of Rybelsus as its direct oral formulation.

Saxenda (liraglutide) (Novo Nordisk) (Commercially Available)	Saxenda, being the CWM counterpart to Victoza, is a past-generation drug with meager weight-loss efficacy and a daily injection requirement. As a result, TRx has largely remained near minuscule levels, hovering below 5,000 TRx for the past two years.	As of Q3 2025, the Gross-to-Net for Saxenda hovered around -40%, interestingly decreasing from around -73% in Q3 2024.	A study analyzing Saxenda persistence rates found that 67.5% persisted for at least 4 months, 53.7% persisted for 6 months, and 27.3% persisted at the 12-month mark.	N/A
Victoza (liraglutide) (Novo Nordisk) (Commercially Available)	Victoza, being a past-generation T2D drug, has seen minuscule TRx over the past 2 years, decreasing from around 10,000 in July 2024 to <5,000 in July 2026. Its share of the US diabetes GLP-1 NBRx market has also shrunk from near 2% in July 2024 to <1% in July 2026.	N/A	Being a relatively dead drug now based on the past-generation active ingredient liraglutide, which has seen LOE across the world including in the US, persistence for the drug is simply not viable, especially with the existence of Rybelsus/Ozempic pill that offers better HbA1c reduction in a more convenient pill format instead of a once-daily injection.	N/A
CagriSema (emaglutide + Cagrilintide): (Novo Nordisk) (Awaiting FDA Approval)	N/A	N/A	N/A	December 2025: FDA New Drug Application for CagriSema 2.4/2.4 mg
Retatrutide (Retatrutide) (Eli Lilly) (Phase 3)	N/A	N/A	N/A	June 2026: TRANSCEND-T2D-1 Phase 3 Trial Results: Retatrutide efficacy for Obese and Overweight with ≥ 1 weight-related comorbidity May 2026: TRIUMPH-1 Phase 3 Trial Results: Retatrutide efficacy for people with T2D
Berobenatide (PF'3944 / MET-097i) (Pfizer) (Phase 3)	N/A	N/A	N/A	February 2026: First results released from VESPER-3, analyzing monthly capabilities of PF'3944 in overweight and obese without T2D

MariTide (maridebart cafraglutide) (Amgen) (Phase 3)	N/A	N/A	N/A	2026: Phase 3 MARITIME 1 and 2 begun to study Obesity and T2D in addition to CV risk, heart failure, and OSA. June 2025: Phase 2 trial result publication outlining weight loss efficacy and HbA1c reduction
Enicepatide (CT-388) (Roche) (Phase 3)	N/A	N/A	N/A	April 2026: Phase 3 trials initiated January 2026: Phase 2 CT388-103 results for obese population weight-loss efficacy
Zenagamtide (Amycretin) (Novo Nordisk) (Phase 3)	N/A	N/A	N/A	May 2026: AMAZE-12 launched, weight maintenance after diet-induced weight loss trial
Survodutide (Boehringer Ingelheim + Zealand Pharma) (Phase 3)	N/A	N/A	N/A	July 2026: SYNCHRONIZE-1 Results released from Phase 3 SYNCHRONIZE-1 and SYNCHRONIZE-MASLD clinical trials on weight loss efficacy and MASH liver fibrosis aiding.
VK2735 (Viking Therapeutics) (Phase 3)	N/A	N/A	N/A	VANQUISH-1 (~4,650) & -2 (~1,000) fully enrolled; maintenance-dosing data expected 3Q26.
Pemvidutide (Altimune) (Transitioning into Phase 3)	N/A	N/A	N/A	July 2026: RECLAIM Phase 2 trial results for heavy drinking days per week reduction December 2025: IMPACT 48-wk MASH and weight loss data

Petrelintide (Zealand Pharma + Roche / Genentech) (Transitioning into Phase 3)	N/A	N/A	N/A	Roche/Zealand deal (~\$1.4–1.65B upfront, 2025); ADA 2026 data; combo w/ Enicepatide (CT-388) trial mid-2026.
Aleniglipron (Structure Therapeutics) (Transitioning into Phase 3)	N/A	N/A	N/A	2026: advancement towards Phase 3 trial initiation in 2H 2026
Elecoglipron (AstraZeneca) (Transitioning into Phase 3)	N/A	N/A	N/A	June 2026: Phase 3 trial initiation started for EMBOLD and ELUMINATE CWM and T2D trials. June 2026: VISTA and SOLSTICE trials concluded
MET-233i (PF'3945) (Pfizer) (Phase 2)	N/A	N/A	N/A	N/A
UBT251 (Novo Nordisk) (Phase 1b/2a)	N/A	N/A	N/A	December 2025: Phase 2 TT2D study completed September 2025: Phase II Chronic Kidney Disease study recruiting started
Xinermei (Mazdutide) (Innovent Biologics + Eli Lilly)	N/A	N/A	N/A	June 2026: Phase 3 GLORY-2 9 mg SC weight loss data released June 2025: First GCG/GLP-1 ever approved (China)

Hemlibra (Emicizumab) (Genentech/Roche) (Commercially Available)	N/A	N/A	N/A	2017: FDA-approved and distributed under the brand name Hemlibra
Fitusiran (Qfitlia) (Sanofi) (Commercially Available)	N/A	N/A	N/A	March 2025: US FDA approval for routine prophylaxis to prevent or reduce bleeding episodes with Hemophilia A or Hemophilia B, with or without factor VIII or IX inhibitors.
Denecimig (Mim8) (Novo Nordisk) (Awaiting FDA Approval)	N/A	N/A	N/A	2026: FRONTIER 2, 3, 4, and 4 extension show positive data for reduced ABR, effectiveness in children ages 1-11, and long-term sustained efficacy and safety Late 2025: Submission to the FDA and EMA June 2025: FRONTIER 5 Trial data shows patients can switch from competitor Emicizumab to Denecimig without a washout period or loading dose, with well-tolerated results and no safety concerns.
Zemocimig (NXT007) (Chigai/Roche) (Transitioning into Phase 3)	N/A	N/A	N/A	May 2026: NXTAGE trial showed positive ABR results and well-tolerated direct switching from Hemlibra to NXT without a washout period.

Rezdiffra (Resmetirom)(Commercially Available)	The first approved MASH drug effectively captured 100% of the US market at its release. Wegovy has started to cannibalize the TRx scripts issued by Hepatologists, as noted in the Wegovy MASH TRx share note.	N/A	N/A	March 2024: FDA approval of Rezdiffra as the first-ever approval for targeting liver fibrosis in MASH patients with noncirrhotic non-alcoholic steatohepatitis
Efruxifermin (Novo Nordisk) (Phase 3)	N/A	N/A	N/A	August 2025: Results from HARMONY 96-week trial on safety and efficacy for people with F2-F4 Fibrosis June 2025: Results from SYMMETRY 96-week trial on safety and efficacy for compensated cirrhosis (F4) patients, showing notable reduction.
Pegozafermin (89bio Pipeline) (Phase 3)	N/A	N/A	N/A	June 2026: ENTRUST Phase 3 trial completed, evaluating prevalence of MASLD and MASH in subjects with SHTG September 2025: Acquired by Roche, which accelerated development for MASH
Efimosfermin (GSK) (Phase 3)	N/A	N/A	N/A	April 2026: FDA grant of breakthrough therapy and EMA Priority Medicines designations for MASH
Ziltivekimab (Novo Nordisk) (Phase 3)	N/A	N/A	N/A	July 2026: ZEUS Phase 3 trial fails primary endpoint in major adverse cardiovascular events (MACE) risk reduction versus placebo in people with atherosclerotic cardiovascular disease (ASCVD), Chronic kidney disease (CKD) and inflammation

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